

DETAINEE DEATH REVIEW: Charles Leo DANIEL, A022745951
Healthcare and Security Compliance Analysis
Tacoma Immigration Processing Center
Tacoma, Washington

As requested by the ICE Office of Professional Responsibility (OPR), External Reviews and Analysis Unit (ERAU), Creative Corrections participated in a review of the death of detainee Charles Leo DANIEL while he was in the custody of the Tacoma ICE Processing Center (TIPC) in Tacoma, Washington. A team consisting of ERAU team lead (b)(6),(b)(7)(C),(b)(7)(F) Inspection and Compliance Specialist, and (b)(6),(b)(7)(C),(b)(7)(F) Inspection and Compliance Specialist, along with Creative Corrections contract personnel (b)(6),(b)(7)(C),(b)(7)(F) Security Subject Matter Expert, and (b)(6),(b)(7)(C),(b)(7)(F) Medical Subject Matter Expert, conducted an on-site review April 2-4, 2024. In addition, on March 29, 2024, the team conducted telephone interviews of TIPC staff not available during the site visit. ERAU requested contractor participation to assess compliance with the applicable ICE 2011 Performance Based National Detention Standards, Revised 2016, governing healthcare and security operations.

This report includes a case synopsis, overview of the facility's healthcare services, detention summary, a narrative overview and chronology of significant events, and conclusions. The information and findings are based on analysis of DANIEL's medical and detention records, video evidence, information reported by staff during interviews, and review of policies and available incident-related documentation.

SYNOPSIS

DANIEL was admitted to TIPC on April 2, 2020. During intake screening, he reported a history of kidney stones and mild hypertension. He also reported he took antidepressant medication, and that he had received counseling, treatment, and medication for mental health problems while in the Washington state correctional system. The medical transfer summary from the Washington Department of Corrections (DOC) validated the information and included a diagnosis of delusional disorder.

DANIEL was on protective custody per his request, for almost the full four years of his detention. TIPC mental health providers diagnosed delusional disorder related to DANIEL's claims the government was electronically monitoring him through air vents and manipulating his health through his food intake and recommended psychiatric medications. DANIEL denied he had delusional disorder and refused recommended psychiatric medications to treat it, but for a time, took antidepressant medication he believed helpful. In addition, medical providers diagnosed uncontrolled hypertension due to consistently elevated blood pressure readings when DANIEL allowed healthcare staff to take them. DANIEL refused medications to control his blood pressure.

DANIEL typically refused to go to the clinic for healthcare appointments on grounds he was distrustful to leave his cell unattended. In August 2022, DANIEL agreed to an examination in the medical clinic for evaluation of a mass on his neck. A computerized tomography (CT) scan of the mass determined it was a cyst, removal of which he refused.

On March 7, 2024, at 10:46 a.m., security staff observed DANIEL partially off his bunk, unresponsive when his name was called. They immediately called for assistance, entered the cell, and confirmed DANIEL was unresponsive. Security and medical staff initiated cardiopulmonary resuscitation (CPR), including use of an automated external defibrillator which administered a shock. Medical and security staff continued CPR until emergency medical services (EMS) arrived. EMS responders assumed care of DANIEL and continued lifesaving measures. At approximately 11:41 a.m., the EMS supervising physician remotely determined lifesaving measures were unsuccessful and instructed EMS staff to terminate their efforts.

DETENTION SUMMARY

DANIEL was classified high custody due to a criminal conviction and incarceration in the state of Washington. Except for the first two weeks of his detention at TIPC, and three days in 2021 when in the medical housing unit, DANIEL was on protective custody in the restricted housing unit per his request. He did not receive any disciplinary reports.

The facility provided recordings of five telephone calls DANIEL made on March 3 and 4, 2024, the last preceding his death on March 7, 2024.¹ All were in English, and he did not mention feeling ill during the calls. He did not receive any visits while at the facility.²

DANIEL submitted numerous informal and formal grievances during his detention. The 22 formal grievances he submitted focused on three topics: harassment by a detention officer, problems receiving mail, and his inability to complete computer science courses and obtain certificates of completion. Staff investigated and found the grievances unfounded. In addition, DANIEL made two allegations that resulted in Sexual Abuse and Assault Prevention and Intervention (SAAPI) investigations. Both were determined unfounded.

HEALTHCARE SERVICES

ICE Health Services Corps (IHSC) officers, GEO staff, and STG International contract employees provide healthcare services. TIPC last earned reaccreditation by the National

¹ See phone calls downloaded. File path: General Records Request – NWIPC (SEA), Calls, 01 Daniel (b)(6),(b)(7)(C) 03-04-24 1944.mp3, 02 Daniel (b)(6),(b)(7)(C) 03-03-24 1509.mp3, 03 Daniel (b)(6),(b)(7)(C) 03-03-24 1458.mp3, 04 Daniel (b)(6),(b)(7)(C) 03-03-24 1447.mp3, 05 Daniel (b)(6),(b)(7)(C) 03-03-24 1436.mp3.

² ERAU interview with COS (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024.

Commission on Correctional Health Care in March 2021.³ Licensure of healthcare personnel involved in DANIEL's care was current.⁴ CPR training of healthcare and security personnel involved in the emergency response was current.⁵ The health services department uses IHSC's electronic medical record system, e-Clinical Works (eCW). Medication administration is documented in a separate electronic system, sMArt, which interfaces with eCW.

NARRATIVE

First month of detention – April 2 – 30, 2020

On April 2, 2020, at 4:30 p.m., DANIEL arrived at TIPC.⁶ (b)(6),(b)(7)(C),(b)(7)(F) RN, completed his medical pre-screening.⁷ RN (b)(6),(b)(7)(C),(b)(7)(F) noted DANIEL spoke English fluently⁸ and that he acknowledged health problems. RN (b)(6),(b)(7)(C),(b)(7)(F) did not identify any urgent healthcare needs.

(b)(6),(b)(7)(C),(b)(7)(F), Detention Officers, completed required intake processes.⁹ (b)(6),(b)(7)(C),(b)(7)(F) Classification Coordinator, classified DANIEL high security.¹⁰

³ See NCCHC Health Services Accreditation Report, dated March 26, 2021. File path: Medical Records Request-NWIPC (SEA), NCCHC, NWIPC 2021 NCCHC Final Report.pdf, pages 1-65.

⁴ See Medical License documents. File path: Medical Records Request - Medical Records Request-NWIPC (SEA), Medical Licenses (b)(6),(b)(7)(C),(b)(7)(F) PSV 5-31-2024.pdf, (b)(6),(b)(7)(C),(b)(7)(F) RN License 06-06-2024.pdf, (b)(6),(b)(7)(C),(b)(7)(F) PA license 7-22-2025.pdf, (b)(6),(b)(7)(C),(b)(7)(F) RN License 01-19-2025.pdf, (b)(6),(b)(7)(C),(b)(7)(F) LPN License 8-24-2024.pdf, (b)(6),(b)(7)(C),(b)(7)(F) PA License 12-18-2025.pdf, (b)(6),(b)(7)(C),(b)(7)(F) LCSW Lic 06.30.2024.pdf, (b)(6),(b)(7)(C),(b)(7)(F) Medical License 05-05-2024.pdf, (b)(6),(b)(7)(C),(b)(7)(F) license 02-08-26.pdf, (b)(6),(b)(7)(C),(b)(7)(F) RN LICENSE 5-20-2024.pdf, (b)(6),(b)(7)(C),(b)(7)(F) PA License 04-11-2022.pdf, (b)(6),(b)(7)(C),(b)(7)(F) PA License 03-12-2026.pdf, (b)(6),(b)(7)(C),(b)(7)(F) Psychologist License 01-15-2025.pdf, (b)(6),(b)(7)(C),(b)(7)(F) SW License 1-26-2025.pdf, (b)(6),(b)(7)(C),(b)(7)(F) Phycologist License 03-31-2026.pdf, (b)(6),(b)(7)(C),(b)(7)(F) MD License 4-30-2024.pdf, (b)(6),(b)(7)(C),(b)(7)(F) MD License 4-30-2024.pdf, (b)(6),(b)(7)(C),(b)(7)(F) RDH License 5-31-2026.pdf.

⁵ See Medical CPR documents. File path: Medical Records Request-NWIPC (SEA), BLS Cards, (b)(6),(b)(7)(C),(b)(7)(F) BLS 05-2025.pdf, (b)(6),(b)(7)(C),(b)(7)(F) BIS 05-2025.pdf, (b)(6),(b)(7)(C),(b)(7)(F) BLS 10-2025.pdf, (b)(6),(b)(7)(C),(b)(7)(F) BLS 3-2025.pdf, (b)(6),(b)(7)(C),(b)(7)(F) BLS 07-2025.pdf, (b)(6),(b)(7)(C),(b)(7)(F) BLS 02-2026.pdf, (b)(6),(b)(7)(C),(b)(7)(F) BLS 07-2024.pdf, (b)(6),(b)(7)(C),(b)(7)(F) BLS 12-2025.pdf. See correctional staff CPR documents. File path: General Records Request-NWIPC (SEA), (b)(6),(b)(7)(C),(b)(7)(F) CPR August 2022.pdf, pages 1-2.

⁶ See Subject Profile. File path: General Records Request – NWIPC, Daniel A022 745 951 Detention File, Daniel 022745951 Sec 1.pdf, page 3.

⁷ See Pre-Screen intake, dated April 2, 2020. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record P19.pdf, pages 424-425.

⁸ English is the national language of Trinidad and Tobago. Based on documentation and interviews, ERAU verified DANIEL spoke English and there were no barriers to communication during his detention.

⁹ See intake documents. File path: General Records Request – NWIPC, Daniel A022 745 951 Detention File, Daniel 022745951 Sec 1.pdf, pages 1-14.

¹⁰ See ICE Custody Classification Worksheet. File path: General Records Request-NWIPC (SEA), Daniel A022 745 951 Detention File, Daniel 022745951 Sec 1.pdf, pages 23-24.

On the same date, (b)(6),(b)(7)(C),(b)(7)(F) RN, completed DANIEL's intake screening.¹¹ RN (b)(6),(b)(7)(C),(b)(7)(F) noted a medical transfer summary¹² arrived with DANIEL and documented the following screening findings:

- DANIEL's vital signs were within normal limits, as follows: temperature 97.7 degrees, pulse 92 beats per minute, respirations 16 breaths per minute, and blood pressure 135/78.¹³ He was 67 inches in height, and he weighed 158.4 pounds. His body mass index (BMI) was normal at 24.81 kg/m.¹⁴
- DANIEL reported his past medical problems included kidney stones and mild hypertension.¹⁵
- DANIEL reported no allergies to medication or food.
- DANIEL denied symptoms of tuberculosis (TB). According to the transfer summary, DANIEL refused a TB skin test, TB screening identified no signs or symptoms of TB.¹⁶
- DANIEL reported he had received counseling and received medication for mental health problems.
- DANIEL reported he was taking fluoxetine¹⁷ for depression, ten milligrams (mg), one capsule by mouth every Monday, Thursday, and Saturday.
- DANIEL denied suicidal or homicidal ideations.
- DANIEL denied having been a victim of physical or sexual assault.
- DANIEL reported using tobacco and marijuana and drinking six alcoholic drinks weekly. He stated he had a previous conviction for driving under the influence of alcohol.

¹¹ See clinical entry titled INTAKE, dated April 2, 2020. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 420-422.*

¹² See Intersystem & Work Release Transfer Summary, dated April 2, 2020. File path: *Medical Records Request-NWIPC (SEA), DANIEL, CHARLES Complete Medical Record Pt10.pdf, pages 469-478.*

¹³ Per American Heart Association (AHA) guidelines, normal blood pressure for adults is a systolic blood pressure <120, and a diastolic blood pressure of <80. Although DANIEL's blood pressure was higher than normal per AHA Guidelines, IHSC's threshold programmed in eCW is higher than AHA's. The eCW system flags readings as abnormal when one or both measurements exceed 140/90. For this report, blood pressure readings are noted as normal unless flagged by eCW or addressed by a practitioner in the clinical entry.

¹⁴ BMI is a measure of body fat based on height and weight and is a good measure of a person's risk for diseases that can occur with being overweight and obesity. A normal BMI is 18.5 to 24.9.

¹⁵ Hypertension is blood pressure exceeding the normal range.

¹⁶ See Intersystem & Medical Transfer Summary, dated April 2, 2020. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf, page 477.*

¹⁷ Fluoxetine is a medication used for depression.

DANIEL signed a medical consent form to receive medical and mental health care.¹⁸ RN (b)(6),(b)(7)(C),(b)(7)(F) offered DANIEL the 2019/2020 influenza vaccination, which he refused.¹⁹ A chest x-ray completed the same day identified no evidence of TB.²⁰

Based on the intake screening findings, RN (b)(6),(b)(7)(C),(b)(7)(F) referred DANIEL to a medical provider and notified (b)(6),(b)(7)(C),(b)(7)(F) Doctor of Psychology (Psy.D.), that DANIEL was diagnosed with depressive disorder, persecuting type²¹ at the Washington State Prison.²² Dr. (b)(6),(b)(7)(C),(b)(7)(F) referred DANIEL to (b)(6),(b)(7)(C),(b)(7)(F) Psy.D., who scheduled him for a mental health appointment.

DANIEL was assigned to general population Unit G, cell 104, at 8:43 p.m.²³

On April 3, 2020, (b)(6),(b)(7)(C),(b)(7)(F) Physician Assistant–Certified (PA-C), completed DANIEL’s initial physical examination and health appraisal.²⁴ She noted the following:

- DANIEL reported excision of an abdominal lipoma²⁵ in January 1997.
- DANIEL denied a history of physical and sexual abuse.
- DANIEL reported he received psychiatric counseling and medication for depression while in state prison.
- DANIEL reported his murder conviction resulted from an act of self-defense.
- DANIEL reported he was being electronically harassed and that people put spirits in him. He stated he heard voices and sees swirling air coming through the vents, commenting officers hear and see it also.
- DANIEL denied thoughts of killing or harming himself or others.
- DANIEL reported an allergy to Risperidone²⁶ with side effect of shortness of breath.

¹⁸ See ICE Health Service Corps Medical Consent Form, signed April 2, 2020. File path: *Medical Records Request-NWIPC (SEA), Forms Consents and Refusals, DANIEL_CHARLES_Treatment Authorization Requests.pdf*, page 4.

¹⁹ See ICE Health Service Corps Refusal Form, dated April 2, 2020. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf*, page 463.

²⁰ See X ray: Chest report, dated April 2, 2020. File path: *Medical Records Request-NWIPC (SEA), Labs and X-Ray, CHARLES DANIELs Lab.X-Ray Results.pdf*, page 1.

²¹ Depression is a mood disorder disorders marked by loss of interest or pleasure in living. A person with persecutory type depression believes a person or group wants to hurt them.

²² See clinical entry titled Telephone Encounter, dated April 2, 2020. File path: *Medical Records Request-NWIPC, (SEA), Mental Health Record, DANIEL CHARLES_Mental Health Records Part 3.pdf*, page 509. Note: The Transfer Summary Problem List Screening and Immunization Record indicates DANIEL has a diagnosis of Delusional Disorder, Persecutory Type, Continuous; the Transfer Summary, Health Care Summary, Active Prescriptions for Current Inmates, has a diagnosis of Depressive Disorder, Not Elsewhere Classified.

²³ See GEO Track housing history. File path: *General Records Request – NWIPC (SEA), Daniel A022 745 951 Housing.pdf*, page 3.

²⁴ See clinical entry titled PE - Initial, dated April 3, 2020. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf*, pages 415-418.

²⁵ Lipoma is a slow-growing, fatty lump that is usually harmless.

²⁶ Risperidone is a medication used to manage psychotic disorders.

- Daniel reported taking the antidepressant medication, fluoxetine every Monday, Thursday, and Saturday.
- DANIEL reported edema²⁷ in his lower extremities and stated he previously used ACE™ elastic bandage wraps for edema in his legs.
- DANIEL denied fever, cough, shortness of breath, dizziness, visual disturbance, weakness, shortness of breath or chest pain.
- DANIEL weighed 156.6 pounds with a BMI of 24.52 kg/m². PA-C [REDACTED] noted his blood pressure of 172/87²⁸ was elevated; other vital signs were within normal limits as follows: temperature 97.7 degrees, pulse 87 beats per minute, and respirations 16 breaths per minute. His visual acuity was 20/15²⁹ in both eyes without glasses.

The general examination findings were normal except for edema in both lower extremities. PA-C [REDACTED] noted DANIEL's speech was rapid, and his thought content was tangential.³⁰ Overall, the examination was abnormal. PA-C [REDACTED] diagnosed unspecified bilateral peripheral edema and a problem related to unspecified psychosocial circumstances. She ordered fluoxetine 10 mg capsule, once a day, but did not obtain specific informed consent for psychotropic medication and discuss possible side effects. She educated DANIEL regarding a healthy lifestyle, dental and personal hygiene, and disease prevention and transmission. PA-C [REDACTED] offered thiazide³¹ to help with leg swelling, but DANIEL declined new medications. He also declined lab tests. PA-C [REDACTED] ordered blood pressure checks, recommended follow-up in one week, and requested previous records from the State of Washington Department of Corrections. She noted DANIEL had already been referred to the mental health department and placed a Medical/Psychiatric Alert³².

On the same date, Dr. [REDACTED] completed a mental health assessment.³³ She noted the following:

- DANIEL denied anxiety but did report he was worried about his case.

²⁷ A local or generalized condition in which body tissues contain an excessive amount of fluid in the interstitial spaces.

²⁸ Per American Heart Association (AHA) guidelines, normal blood pressure for adults is a systolic blood pressure <120, and a diastolic blood pressure of <80. Although DANIEL's blood pressure was higher than normal per AHA Guidelines, IHSC's threshold programmed in eCW is higher than AHA's. The eCW system flags readings as abnormal when one or both measurements exceed 140/90. For this report, blood pressure readings are noted as normal unless flagged by eCW or addressed by the practitioner in the clinical entry.

²⁹ Visual acuity of 20/15 is sharper than average.

³⁰ Tangential meaning speaking about topics unrelated to the main topic of discussion.

³¹ Thiazide is a type of diuretic drug that increases urine flow and reduces fluid volume and blood pressure.

³² A Medical/Psychiatric Alert prompts medical clearance prior to transfer or removal from TIPC. See ICE Health Service Corps Medical/Psychiatric Alert form, dated April 3, 2020. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf, page 465.

³³ See clinical entry titled MH Intake, dated April 3, 2020. File path: Medical Records Request-NWIPC (SEA), Mental Health Record, DANIEL_CHARLES_Mental Health Records Part 3.pdf, 505-507.

- DANIEL denied auditory and visual hallucinations, but remarked, “It doesn’t have to be heard, its [*sic*] auditory perception. It’s like electronic harassment.” He described paranoid delusions, stating the electronic harassment comes through the air vents.
- DANIEL denied suicidal thoughts and difficulty sleeping.
- DANIEL reported previous mental health treatment and said he was involuntarily admitted to the Special Offender Center in prison. He was there from 2006 to 2009, and again from 2016 to November 2018. He said he went to Washington State Penitentiary in Walla Walla, Washington, in 2019.
- DANIEL reported taking fluoxetine since 2018.
- DANIEL reported he smoked marijuana a few times a week when 20 to 40 years old and tried cocaine two times. He drank alcohol two to three times a week but only one drink.

Dr. (b)(6),(b)(7)(C),(b)(7)(F) deferred a mental health diagnosis. Her plan was for DANIEL to remain in general population, follow up with a provider in four to five weeks or sooner if necessary, and refer to the psychiatrist for medication management. She instructed DANIEL on how to access emergency services if needed and discussed hygiene, relaxation skills, stress management skills, and coping. DANIEL verbalized understanding and agreed with the treatment plan.

On April 4, 2020, (b)(6),(b)(7)(C),(b)(7)(F) RN, checked DANIEL’s blood pressure per PA-C (b)(6),(b)(7)(C),(b)(7)(F) order.³⁴ She recorded three readings from an automated blood pressure device, all of which exceeded IHSC’s threshold for abnormal blood pressure programmed in eCW: 154/84, 149/86 and 154/84. DANIEL refused to have his blood pressure taken manually. He denied chest pain, shortness of breath, or weakness and reported he felt good. RN (b)(6),(b)(7)(C),(b)(7)(F) informed (b)(6),(b)(7)(C),(b)(7)(F) PA-C, of the blood pressure readings but did not receive new orders.³⁵

On April 6, 2020, (b)(6),(b)(7)(C),(b)(7)(F) Commander (CDR), Advanced Registered Nurse Practitioner (ARNP), assessed DANIEL for hypertension.³⁶ DANIEL’s vital signs were within normal limits except for an elevated blood pressure at 160/97. ARNP (b)(6),(b)(7)(C),(b)(7)(F) noted DANIEL’s blood pressure was classified as stage 2 hypertension with poor control according to IHSC’s clinical guidelines for hypertension.³⁷ DANIEL reported fluid in his legs and denied chest pain. ARNP (b)(6),(b)(7)(C),(b)(7)(F) discussed the blood pressure readings with DANIEL; also, the importance of pharmacological treatment to reduce blood pressure and decrease risk for organ damage and cardiovascular events. DANIEL stated to ARNP (b)(6),(b)(7)(C),(b)(7)(F) there were external forces and electrical impulses

³⁴ See clinical entry titled 1. BP CHECKS X3, dated April 4, 2020. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 408-409.

³⁵ See Adult Acute Care (Sick Call) Clinical Nursing Guidelines v6 9-2023. File path: Medical Records Request-NWIPC (SEA), Policies, IHSC Adult Care Sick Call Clinical Nursing Guidelines v6 FINAL 9-30-23 Signed.pdf, page 121.

³⁶ See clinical entry titled HTN, dated April 6, 2020. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 405-406.

³⁷ See Providers Clinical Guidelines: HYPERTENSION, dated May 25, 2017. File path: Medical Records Request-NWIPC (SEA), Policies, IHSC Hypertension Guidelines_May 2017.docx, page 5.

causing his elevated blood pressure.³⁸ NP [REDACTED] offered DANIEL several medication options, but he was only willing to try chlorthalidone³⁹. NP [REDACTED] diagnosed essential hypertension and prescribed chlorthalidone 25 mg, one tablet daily. DANIEL would not agree to lab tests.

Although DANIEL agreed to take chlorthalidone, he refused the medication from April 9 through 13, 2020.⁴⁰ According to the refusal forms, he stated the medication was making him worse. [REDACTED], Licensed Practical Nurse (LPN), notified his provider.

On April 14, 2020, ARNP [REDACTED] assessed DANIEL for noncompliance with chlorthalidone.⁴¹ DANIEL's blood pressure was elevated at 150/82 at time of the encounter. He stated he did not need to be treated pharmacologically for high blood pressure; also, that he took the medication for three days and his blood pressure went up. Between ARNP [REDACTED] order for chlorthalidone on April 6, 2020, and the encounter this date, the medical record documents the following:

- On April 8, 2020, DANIEL's blood pressure was elevated at 171/88 when checked by [REDACTED], RN.⁴² DANIEL refused to have blood pressure manually rechecked, stating he was being electronically harassed.
- On April 10, 2020, DANIEL's blood pressure was elevated at 152/76 when checked by RN [REDACTED] with an automated device and 120/80 when taken manually.⁴³
- On April 12, 2020, DANIEL's blood pressure was elevated at 157/85 when checked by RN [REDACTED].⁴⁴

ARNP [REDACTED] discussed the risks of not treating hypertension, including organ damage and death. DANIEL voiced understanding and ARNP [REDACTED] stopped chlorthalidone per DANIEL's request.

³⁸ See clinical entry titled HTN, dated April 6, 2020. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 405-406.*

³⁹ Chlorthalidone is a diuretic which reduces the amount of water in the body by increasing the flow of urine, which helps to lower blood pressure.

⁴⁰ See ICE Health Service Corps Refusal Forms, dated April 9, 2022, through April 14, 2020. *File path: Medical Records Request-NWIPC (SEA), Refusals, DANIEL, CHARLES Refusals.pdf, pages 115-121.*

⁴¹ See clinical entry titled 1. Noncompliance with medications, dated April 14, 2020. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 395-396.*

⁴² See clinical entry titled BP Check per [REDACTED] dated April 8, 2020. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 403-404.*

⁴³ See clinical entry titled BP Check per [REDACTED] dated April 10, 2020. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 400-401.*

⁴⁴ See clinical entry titled BP Check per [REDACTED] dated April 12, 2020. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 397-398.*

On April 16, 2020, DANIEL was moved to Unit H, TIPC's Restricted Housing Unit (RHU).⁴⁵ On the administration segregation order, Lieutenant [REDACTED] documented DANIEL requested protective custody for fear of others' safety; also, because he believed the government was electronically monitoring him.⁴⁷

RN [REDACTED] completed DANIEL's pre-segregation clearance, noting his vital signs were within normal limits, as follows: temperature 97.7 degrees, pulse 90 beats per minute, respirations 16 breaths per minute, oxygen saturation of 95 percent and blood pressure 138/70.⁴⁸ DANIEL denied thoughts of harming himself or others and did not have any medical complaints. RN [REDACTED] informed DANIEL how to access medical, mental health and dental care while in the RHU.

Except for June 23–25, 2021, when DANIEL was assigned to mental health observation (see below), he remained housed in the RHU on protective custody from April 12, 2020, through the date of this death, March 7, 2024. He was housed alone for the full term of detention. The following describes security and healthcare staff requirements for routine monitoring of detainees assigned to the RHU, and summarizes compliance with respect to DANIEL.

- There are two officers assigned to the RHU on each shift. Officers are required to conduct health and welfare checks and personally observe each detainee on an irregular schedule a minimum of every 30 minutes. Officers document their rounds in a logbook⁴⁹ and electronically using the [REDACTED] (b)(7)(E) system.⁵⁰ As noted below, officers conducted required rounds preceding his death on March 7, 2023.
- Officers record activities on logs maintained at the RHU Officers' station. According to DANIEL's record and staff interviews, he was offered recreation and showers; however, he generally refused.⁵² [REDACTED] Detention Officer, stated that during his tenure as

⁴⁵ See GEO Track housing history. File path: *General Records Request – NWIPC (SEA), Daniel A022 745 951 Housing.pdf, page 3.*

⁴⁶ Lieutenant [REDACTED] first name is not available. He is no longer employed at TIPC.

⁴⁷ See Administrative Segregation Order, dated April 16, 2020. File path: *General Records Request – NWIPC (SEA), Daniel A022 745 951 seg 1.pdf, page 1.*

⁴⁸ See clinical entry titled Clearance, dated April 16, 2020. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 392-393.*

⁴⁹ See RHU Logbook. File path: *General Records Request – NWIPC (SEA), Logbooks, RHU Logbook entries 3-7-24 - Old Book.pdf.*

⁵⁰ The [REDACTED] (b)(7)(E) system is an electronic, handheld scanner designed to electronically record health and welfare rounds in the SMU. Staff touch the end of [REDACTED] (b)(7)(E) to the corresponding button on each SMU door. Once the connection is made correctly, the [REDACTED] (b)(7)(E) will emit a beep and a red light shows on the pipe.

⁵¹ See GEO tour hourly log. File path: *General Records Request – NWIPC (SEA), GEO TIME LOGS.pdf.*

⁵² See Detainee file. File path: *General Records Request – NWIPC (SEA), Daniel A022 745 951 Detention File, Daniel 022745951 seg 1.pdf, seg 2.pdf, seg 4.pdf, seg 6.pdf, seg 7.pdf, seg 11.pdf, and seg 12.pdf. Also, ERAU interview with [REDACTED] (b)(6),(b)(7)(C),(b)(7)(F), Chief of Security (COS), [REDACTED] (b)(6),(b)(7)(C),(b)(7)(F), Lieutenant, and [REDACTED] (b)(6),(b)(7)(C),(b)(7)(F) [REDACTED] (b)(6),(b)(7)(C),(b)(7)(F) Detention Officers, on March 29 and April 2 and 4, 2024.*

an RHU officer DANIEL never left his cell for a shower or recreation.⁵³ He said DANIEL did not like to leave his cell because he did not want staff to search or steal his belongings; specifically, his paperwork. Staff reported that DANIEL maintained his personal hygiene by taking what they referred to as birdbath showers⁵⁴ in his cell. DANIEL accepted meals and based on the documentation and staff reports, ate them.

- Security staff conduct weekly in-person interviews with detainees to discuss their segregation status. DANIEL's record documents security staff completed required status reviews, during which he consistently requested continuation on protective custody.⁵⁵
- Healthcare requirements for monitoring detainees in the RHU include daily rounds by RNs, during which they ask detainees if they have complaints or concerns, thoughts of self-harm, and if they wish to see medical. In addition, mental health providers are required to conduct weekly face-to-face rounds to monitor mental health status, identify signs of deterioration, and recommend additional treatment. DANIEL's medical record documents compliance with nursing and mental health provider rounds.⁵⁶
- In addition to status reviews by security personnel and rounds by healthcare professionals, a multi-disciplinary committee reviews detainees in the SMU on a weekly basis. The multi-disciplinary committee includes a representative from facility leadership, medical and mental health professionals, and security staff. ICE/ERO staff may participate in the committee meetings in person or virtually. The committee reviews the detainee's current behavior and physical and mental health and considers whether a change in status is warranted. Detainees are informed of committee recommendations and afforded an opportunity to provide input. DANIEL's records document weekly multi-disciplinary committee meetings during the six months preceding his death, with no recommendation for change.⁵⁷

On April 21, 2020, PA-C (b)(6),(b)(7)(C),(b)(7)(F) conducted an encounter with DANIEL to discuss his refusal of fluoxetine doses.⁵⁸ DANIEL stated he refused the medication because he wanted fluoxetine on Mondays, Thursdays, and Saturdays rather than daily as prescribed. He stated he needed the

⁵³ ERAU interview with DO (b)(6),(b)(7)(C),(b)(7)(F) on March 29, 2024.

⁵⁴ A birdbath shower or bird bathing is washing oneself with a washcloth or the corner of one's towel in the sink.

⁵⁵ See Administrative Detention Reviews, File Path: General Records Request – NWIPC (SEA), Daniel, Charles Leo A022 745 951, Detention File.

⁵⁶ See Full Medical Record Pt1 – Pt10. File path: Medical Records Request – NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt1-10.pdf.

⁵⁷ See Weekly multi-disciplinary meeting notes. File path: General Records Request – NWIPC (SEA), RHU 6 months of weekly multi-disciplinary committee reviews.

⁵⁸ See clinical entry titled SC: medication s/e, refill, dated April 21, 2020. File path: Medical Records Request- NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt 9.pdf, pages 379-380. The MAR shows he refused fluoxetine on April 14, 2020, and April 19, 2020.

medication to keep his clarity; also, that he believed the medication was being kept from him so he will not be clear for his court date next week. DANIEL denied audio and visual hallucinations and suicidal or homicidal ideations. DANIEL again refused medication for hypertension, stating chlorthalidone had increased his blood pressure to 200. He stated he cannot take anything to decrease his blood pressure because he will have nothing left to fight the electronic harassment to which he is subject every day. DANIEL reported edema in his lower legs but said he believed the edema in his legs had improved since he had been washing his food. DANIEL denied chest pain, palpitations, or shortness of breath. DANIEL's vital signs were normal except for high blood pressure of 172/99. DANIEL continued to endorse fear of taking medications for blood pressure. PA-C (b)(6),(b)(7)(C),(b)(7)(F) did not attempt starting treatment because of DANIEL's state of mind and noted he had an appointment with mental health the following day.

On April 22, 2020, Dr. (b)(6),(b)(7)(C),(b)(7)(F), psychiatrist, evaluated DANIEL⁵⁹ Dr. (b)(6),(b)(7)(C),(b)(7)(F) noted the following:

- DANIEL reported he came to ICE custody from the Washington State correctional system where he had been incarcerated for approximately 20 years.
- DANIEL reported he was diagnosed with delusional disorder and medicated with risperidone by force many years ago for a short period of time. He said he did not agree with the diagnosis and refused medications.
- DANIEL stated he had proof of property theft and persecution while in the Washington State custody.
- DANIEL reported he was harassed by electronic monitoring and special waves were targeting him. He also felt the food in the system had a special ingredient to lower his energy and he counteracted it by taking fluoxetine three times a week.
- DANIEL asked to continue the fluoxetine but wanted it only on Monday, Thursday, and Saturday.
- DANIEL refused a base line lab/blood test, stating they may inject poison in him as they did while he was in the state prison.

Dr. (b)(6),(b)(7)(C),(b)(7)(F) noted fluoxetine is not indicated to treat delusional thinking, but he would continue the medication since DANIEL had had taken it for two years. Dr. (b)(6),(b)(7)(C),(b)(7)(F) also noted DANIEL felt distressed and depressed, and continuation of fluoxetine may help with the secondary depression. Dr. (b)(6),(b)(7)(C),(b)(7)(F) prescribed fluoxetine 10 mg tablets three times a week on Monday,

⁵⁹ See clinical entry titled Pt came to facility with psychotropic medication, dated April 22, 2020. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 372-373.*

Thursday, and Saturday. DANIEL signed a consent form for the medication.⁶⁰ Dr. [REDACTED] noted follow up in four to six weeks.⁶¹

On April 29, 2020, during an encounter with PA-C [REDACTED] DANIEL requested a magnet and sea salt to ward off the spirits and block electronic harassment.⁶² He also requested a sperm and fecal test to find out the poisons attacking him. His vital signs were normal except for high blood pressure of 168/108. DANIEL continued to refuse medications to control blood pressure because he believed the medications may contain poison. PA-C [REDACTED] again discussed risks of hypertension, including myocardial infarction, stroke, and kidney failure. She informed DANIEL that sea salt and magnets could not be prescribed, and he did not meet criteria for sperm and fecal tests.

May 1 – December 31, 2020

Security Summary

Security personnel conducted required status reviews during which DANIEL requested continued protective custody.

Healthcare Summary

Nurses and mental health professionals documented required rounds. As shown in the below table, DANIEL refused care on multiple occasions during his first month of detention and the remainder of the calendar year, including medications, hypertension chronic care follow-up, vital signs, and medical and mental health provider appointments.

DATE	CARE REFUSED	SIGNED REFUSAL FORM
4/2/2020	Vaccine	Yes
4/6/2020	HTN baseline lab work	Yes

⁶⁰ See ICE Health Services Corps consent for Psychotropic Medications for Adults form, dated April 22, 2020. *File path: Medical Records Request-NWIPC (SEA), Treatment Authorization, DANIEL_CHARLES_Treatment Authorization Requests.pdf, page 3.*

⁶¹ See clinical entry titled Pt came to facility with psychotropic medication, dated April 22, 2020. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, page 373.*

⁶² See clinical entry titled SC: multiple requests, dated April 29, 2020. *File path: File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 352-353.*

4/9/2020	Medication: chlorthalidone	Yes
4/10/2020	Medication: chlorthalidone	Yes
4/11/2020	Medication: chlorthalidone	Refused
4/12/2020	Medication: chlorthalidone	Yes
4/13/2020	Medication: chlorthalidone	Refused
4/14/2020	Medication: floxetine	Refused
4/14/2020	Medication: chlorthalidone	Refused
4/14/2020	Medication: hypertension (not specified)	Yes
4/19/2020	Medication: floxetine	Yes
5/2/2020	Lab work	Refused
5/5/2020	Provider appointment	Refused
5/20/2020	Hypertention appointment	Refused
5/21/2020	Chronic care appointment	Refused
5/22/2020	Grievance and chronic care appointment	Refused
5/26/2020	Provider appointment	Refused
6/2/2020	COVID 19 lab work	Yes
6/25/2020	Chronic care appointment	Refused
6/26/2020	Chronic care appointment	Refused
7/22/2020	Chronic care appointment	Refused
8/20/2020	Provider appointment	Refused
9/17/2020	Vaccine	Yes
10/20/2020	Mental health evaluation	Refused
11/2/2020	Medication: cholecalciferol	Refused
11/3/2020	Vaccine	Yes
11/11/2020	Provider appointment	Refused
11/12/2020	Provider appointment	Refused
11/17/2020	Provider appointment	Refused
12/1/2020	Mental health evaluation	Refused
12/12/2020	Medication: Citalopram	Refused
12/13/2020	Medication: Citalopram	Refused
12/15/2020	Medication: Citalopram	Refused

As described above, DANIEL's blood pressure readings during the first month of his detention were usually elevated. The following table shows his blood readings from the time of his admission on April 2, 2020, through the end of calendar year.

DATE	BLOOD PRESSURE READING
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4/2/2020	135/78
4/3/2020	172/87
4/4/2020	sitting:154/84; automated:149/86; repeat:154/84
4/6/2020	160/97
4/8/2020	171/88
4/10/2020	automated:152/76; manual:120/80
4/12/2020	157/85
4/14/2020	150/82
4/16/2020	138/70
4/21/2020	172/99
4/29/2020	168/108
5/1/2020	159/102
7/25/2020	155/86
7/30/2020	160/84
7/31/2020	178/83
8/5/2020	185/99
8/21/2020	152/98
10/26/2020	157/88
11/19/2020	181/113

Significant Healthcare Encounters

On May 1, 2020, DANIEL submitted a sick call request in which he stated he had blood in his urine.⁶³ (b)(6),(b)(7)(C),(b)(7)(F) PA-C, assessed DANIEL and performed a urine dipstick test⁶⁴ which confirmed the presence of blood.⁶⁵ DANIEL reported no symptoms. His blood pressure was elevated at 159/102 and he expressed delusional thoughts. DANIEL stated he was being tortured at TIPC, had toxic substances put into his body and believed sperm and fecal tests were needed to prove his case. He also stated harmful radio waves and extrinsic elements were entering his body through the air vents in his cell and the food he eats. PA-C (b)(6),(b)(7)(C),(b)(7)(F) determined DANIEL had hematuria⁶⁶ and continued the diagnosis of essential hypertension. He ordered a routine

⁶³ See clinical entry titled RHU ROUNDS, dated May 1, 2020. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf*, pages 348. Scanned Sick Call form completed by RN (b)(6),(b)(7)(C),(b)(7)(F) was not located in the medical file.

⁶⁴ Test of urine using a special strip of paper that is dipped into a sample of urine. The result is available almost immediately. Also called rapid urine test.

⁶⁵ See clinical entry titled SC: Genitourinary-blood in urine, dated May 1, 2020. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf*, pages 345-346.

⁶⁶ Hematuria is blood in the urine.

urinalysis and culture⁶⁷ for the following day and scheduled a follow-up appointment in one week. PA-C (b)(6),(b)(7)(C),(b)(7)(F) encouraged DANIEL to adequately hydrate and avoid activities that may aggravate hematuria, and to agree to treatment for hypertension.

DANIEL refused the urinalysis with culture test ordered by PA-C (b)(6),(b)(7)(C),(b)(7)(F)⁶⁸ DANIEL also refused his scheduled follow-up appointment with PA-C (b)(6),(b)(7)(C),(b)(7)(F)⁶⁹

On May 12, 2020, the Mental Health Treatment Team consisting of Dr. (b)(6),(b)(7)(C),(b)(7)(F), Dr. (b)(6),(b)(7)(C),(b)(7)(F) and Dr. (b)(6),(b)(7)(C),(b)(7)(F) met to discuss DANIEL.⁷⁰ The Team discussed his symptoms and changed his diagnosis to delusional disorder. During mental health rounds the next day, Dr. (b)(6),(b)(7)(C),(b)(7)(F) documented DANIEL was not exhibiting active symptoms and did not meet IHSC criteria for a patient with serious mental illness (SMI).⁷¹

On June 22, 2020, Dr. (b)(6),(b)(7)(C),(b)(7)(F) saw DANIEL in the RHU.⁷² DANIEL continued to report harassment by electronic monitoring and that special waves were targeting him. DANIEL claimed sometimes there is an ingredient in the food served to him that lowers his energy. DANIEL refused to accept the diagnosis of delusional disorder and did not want any medication such as Risperidone, stating the medication is like poison to his brain.

On July 21, 2020, Dr. (b)(6),(b)(7)(C),(b)(7)(F) saw DANIEL in the RHU.⁷³ DANIEL again reported harassment by electronic monitoring and that special waves were targeting him. DANIEL requested an increase in the dosage of fluoxetine to counteract his low mood and energy. Dr. (b)(6),(b)(7)(C),(b)(7)(F) increased the fluoxetine dosage from ten to 20 mg, one capsule three times a week on Monday, Thursday, and Saturday, as needed. DANIEL continued to refuse psychotropic medication for the delusional disorder diagnosis. Dr. (b)(6),(b)(7)(C),(b)(7)(F) noted that even though fluoxetine was not indicated to treat delusional thinking, he would continue the prescription since DANIEL had taken it for more than two years and it seemed to help his secondary depression.

⁶⁷ Urine culture test checks for germs that cause infections.

⁶⁸ See ICE Health Service Corps Refusal Form, dated May 2, 2020. File path: *Medical Records Request-NWIPC (SEA), Refusals, DANIEL CHARLES Refusals.pdf*, page 106.

⁶⁹ See ICE Health Service Corps Refusal Form, dated May 5, 2020. File path: *Medical Records Request-NWIPC (SEA), Refusals, DANIEL CHARLES Refusals.pdf*, page 104.

⁷⁰ See clinical entry titled Consultation with MH Treatment Team, dated May 12, 2020. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf*, pages 312-313.

⁷¹ See clinical entry titled MH Rounds, dated May 13, 2020. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf*, pages 315-316.

⁷² See clinical entry titled Delusional Disorder, dated June 22, 2020. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf*, pages 198-199.

⁷³ See clinical entry titled Delusional Thinking, dated July 21, 2020. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf*, pages 121-122.

On July 25, 2020, DANIEL complained of severe, aching stomach pain which radiated to his back.⁷⁴ He rated his pain level ten of ten⁷⁵. DANIEL reported the pain began after eating dinner; also, that he had belched frequently and vomited at least seven times in the sink.

(b)(6),(b)(7)(C),(b)(7)(F) RN, assessed DANIEL and noted his abdomen was not distended and he denied diarrhea. Per Clinical Nursing Guidelines for Heartburn/Epigastric pain,⁷⁶ RN (b)(6),(b)(7)(C),(b)(7)(F) gave DANIEL 30 milliliters (ml) of aluminum-magnesium-simethicone suspension⁷⁷, for an upset stomach, one time dose. RN (b)(6),(b)(7)(C),(b)(7)(F) called PA-C (b)(6),(b)(7)(C),(b)(7)(F) and received a telephone order to give DANIEL a one-time dose of acetaminophen⁷⁸ three 325 mg tablets, for pain. DANIEL's vital signs were within normal limits except for elevated blood pressure of 155/86. DANIEL was briefly placed in a holding cell in the medical unit for monitoring and then was escorted back to his cell later the same day.

On July 30, 2020, PA-C (b)(6),(b)(7)(C),(b)(7)(F) evaluated DANIEL.⁷⁹ He noted DANIEL had refused his follow-up chronic care appointment and chlorthalidone for hypertension. DANIEL's blood pressure was elevated at 160/84. DANIEL specifically requested plain, non-coated, aspirin for pain. He said he would not use aspirin more than twice a week and agreed to acetaminophen as well. PA-C (b)(6),(b)(7)(C),(b)(7)(F) ordered aspirin EC tablet delayed release, 325 mg, one tablet once a day as needed for 30 days, and acetaminophen tablet 325 mg, two tablets three times a day as needed for three days. PA-C (b)(6),(b)(7)(C),(b)(7)(F) scheduled a follow-up appointment in four weeks.

On July 31, 2020, PA-C (b)(6),(b)(7)(C),(b)(7)(F) saw DANIEL in the RHU to discuss his uncontrolled hypertension.⁸⁰ She noted DANIEL's blood pressure was 178/83; all other vital signs were normal. DANIEL admitted he had swelling in his legs but refused anti-hypertensive medications. He stated he continued to experience electronic harassment which he believed affected his blood pressure. PA-C (b)(6),(b)(7)(C),(b)(7)(F) encouraged DANIEL to consider alternative blood pressure medications and discussed the risks of uncontrolled hypertension, including risk of

⁷⁴ See clinical entry titled Stomach Pain, dated July 25, 2020. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 105-106.

⁷⁵ Pain level is based on the standardized pain scale of zero to ten measuring a patient's self-report of pain. Zero is no pain and ten is the worst pain ever experienced.

⁷⁶ See EPIGASTRIC PAIN (MILD)/BLOATING/GAS(Gastritis Heartburn) RN guideline. File path: Medical Records Request-NWIPC (SEA), Policies, CNG 2023-Epigastric Pain Bloating.pdf, page 1-2.

⁷⁷ Medication used to treat the symptoms of too much stomach acid in the stomach and frequently used to relieve symptoms of extra gas such as belching, bloating, and feeling of pressure in the stomach.

⁷⁸ Acetaminophen is used to treat mild to moderate pain and is more commonly known by the brand name Tylenol.

⁷⁹ See clinical entry titled Med Request, dated July 30, 2020. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 85-86.

⁸⁰ See clinical entry titled HTN, address letter, dated July 31, 2020. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 80-82.

myocardial infarction⁸¹, stroke⁸², and transient ischemic attack (TIA)⁸³. DANIEL refused to try other medications.

On August 5, 2020, PA-C [REDACTED] evaluated DANIEL for complaint of mild epigastric symptoms of gas and dyspepsia⁸⁴ for two weeks.⁸⁵ DANIEL's vital signs were within normal limits except for an elevated blood pressure of 185/99. PA-C [REDACTED] prescribed bismuth subsalicylate 262 mg chewable tablet, two tablets by mouth, three times a day as needed. DANIEL again refused medication to treat hypertension. PA-C [REDACTED] encouraged DANIEL to moderately exercise for at least 30 minutes a day, three to four times a week, make healthy dietary choices, and drink at least eight to ten glasses of water a day. PA-C [REDACTED] educated DANIEL on the benefits of reduced salt intake and provided education to reduce epigastric symptoms by avoiding coffee and spicy food, caffeine, and citrus.

On August 21, 2020, PA-C [REDACTED] saw DANIEL for a hypertension follow-up appointment.⁸⁶ DANIEL denied symptoms of hypertension and again refused anti-hypertensive medications and lab tests. DANIEL's vital signs were within normal limits except for elevated blood pressure of 152/98. PA-C [REDACTED] classified the severity of disease as Stage 1 (140-159/90-99) and determined the degree of control, based on hypertension without diabetes, was fair (140-160/90-100).⁸⁷

On September 15, 2020, Dr. [REDACTED] saw DANIEL in the RHU.⁸⁸ He noted that for the last few weeks DANIEL had refused fluoxetine⁸⁹. DANIEL reported that unless the fluoxetine came from his personal property, he would refuse it. He stated he believed the facility's supply of fluoxetine was tainted and contaminated, it did not help his energy level, and did not decrease inflammation. DANIEL said no one understood the level of persecution he felt and strongly believed that all his activity and thoughts were monitored by the government, by high frequency

⁸¹ Myocardial infarction occurs when the flow of blood to the heart is blocked. The blockage is most often due to the formation of plaque in the coronary arteries that supply the heart. This interruption of blood flow can damage or destroy part of the heart muscle.

⁸² Stroke occurs when the blood supply to part of the brain is interrupted or reduced, preventing brain tissue from getting oxygen and nutrients.

⁸³ TIA is a short period of symptoms similar to those of a stroke. It is caused by a brief blockage of blood flow to the brain.

⁸⁴ Dyspepsia is an upset stomach.

⁸⁵ See clinical entry titled SC: Gastrointestinal-gas, dated August 51, 2020. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 64-65.

⁸⁶ See clinical entry titled 3mth HTN rescheduled, dated August 21, 2020. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt9.pdf, pages 14-15.

⁸⁷ See Providers Clinical Guidelines: HYPERTENSION, dated May 25, 2017. File path: Medical Records Request-NWIPC (SEA), Policies, IHSC Hypertension Guidelines May 2017.docx, page 5.

⁸⁸ See clinical entry titled Delusional Disorder, dated September 16, 2020. File path: Medical Records Request-NWIPC (SEA), Mental Health Record, DANIEL CHARLES Mental Health Records Part 3.pdf, pages 402-403.

⁸⁹ The MAR shows he refused fluoxetine offered on September 5, 7, 10, 12, and 14 2020. See sMARt MAR, dated September 2021. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf, page 366.

waves. DANIEL again stated he did not accept the diagnosis of delusional disorder and did not want to take medication to treat it. Dr. [REDACTED] discontinued the fluoxetine prescription and scheduled a follow-up in one month or as needed.

On October 20, 2020, DANIEL refused his mental health evaluation with Dr. [REDACTED]⁹⁰ Dr. [REDACTED] determined routine psychiatric follow-up was no longer necessary since DANIEL did not accept his mental illness and need for medication, nor was he taking any.⁹¹ Dr. [REDACTED] noted no further appointments would be scheduled but should the need arise, DANIEL could be referred for psychiatric services.

On October 26, 2020, Dr. [REDACTED] spoke with DANIEL in the RHU pursuant to DANIEL's request to see him⁹² to discuss the medication, citalopram⁹³. Dr. [REDACTED] noted fluoxetine was discontinued on September 15, 2020, and DANIEL was again experiencing tiredness, low energy, and inability to focus. DANIEL stated he was stressed about an upcoming court hearing and that coping with what he believed was persecution made him depressed. DANIEL again would not accept delusional disorder as a diagnosis and did not want any medications to treat it. DANIEL's vital signs were normal except for an elevated blood pressure of 157/88. Dr. [REDACTED] prescribed citalopram, 10 mg tablet, one tablet every day, and cholecalciferol⁹⁴, one 2000 IU tablet every day. DANIEL signed a consent form for the citalopram.⁹⁵ Dr. [REDACTED] scheduled a follow-up appointment in one week.

On November 3, 2020, Dr. [REDACTED] met with DANIEL in the RHU.⁹⁶ DANIEL believed the cholecalciferol supplied by the pharmacy tasted funny and was not agreeing with him. He feared it was contaminated. DANIEL reported citalopram improved his tiredness, low energy, and inability to focus. Dr. [REDACTED] continued the citalopram and discontinued the cholecalciferol. He started DANIEL on a multivitamin tablet, one tablet daily.

⁹⁰ See ICE Health Service Corps Refusal Form, dated October 20, 2020. *File path: Medical Records Request-NWIPC (SEA), Refusals, DANIEL, CHARLES Refusals.pdf, page 84.*

⁹¹ See clinical entry titled Paranoid Delusions, dated October 20, 2020. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt8.pdf, pages 352-353.*

⁹² See Patient is requesting to take Celexa, dated October 26, 2020. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt8.pdf, pages 336-337.*

⁹³ Citalopram, commonly known by the brand name Celexa, is a medication used to treat depression.

⁹⁴ Cholecalciferol is a form of Vitamin D.

⁹⁵ See CONSENT TO USE OF SEROTONIN REUPTAKE INHIBITOR ANTIDEPRESSANT MEDICATION form, dated October 26, 2020. *File path: Medical Records Request-NWIPC (SEA), Treatment Authorization, DANIEL CHARLES Treatment Authorization Requests.pdf page 2.*

⁹⁶ See clinical entry titled "Feels stressed out", dated November 3, 2020. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt8.pdf, pages 310-311.*

On November 19, 2020, PA-C (b)(6),(b)(7)(C),(b)(7)(F) met with DANIEL regarding a recent grievance and obtained a blood pressure reading of 181/113.⁹⁷ DANIEL refused any medications for hypertension and refused labs.

On December 1, 2020, DANIEL refused a psychiatric follow-up appointment with Dr. (b)(6),(b)(7)(C),(b)(7)(F).⁹⁸ Dr. (b)(6),(b)(7)(C),(b)(7)(F) noted DANIEL was medication compliant and citalopram and the multivitamin would continue.⁹⁹ He scheduled a follow-up appointment in one month.

On December 15, 2020, Dr. (b)(6),(b)(7)(C),(b)(7)(F) discontinued citalopram.¹⁰⁰ DANIEL had refused the medications on December 12, 13, and 15, 2020.

January 1, 2021 – December 31, 2021

Security Summary

Except for two days, June 23-25, 2021, DANIEL remained on protective custody. Security personnel conducted required status reviews, during which DANIEL requested continuation on protective custody.

From June 23 – 25, 2021, DANIEL was assigned to mental health observation in the medical housing unit. During those two days, officers documented required 15-minute checks.¹⁰¹

Healthcare Summary

Nurses and mental health professionals continued to document required rounds.

As shown below, DANIEL refused to have blood drawn for lab work, annual physical examination, hypertension chronic care clinic appointments, vital signs, and medical and mental health provider appointments.

⁹⁷ See clinical entry titled Grievance, dated November 19, 2020. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt8.pdf, pages 259-260.

⁹⁸ See ICE Health Service Corps Refusal Form, dated December 1, 2020. File path: Medical Records Request-NWIPC (SEA), Refusals, DANIEL_CHARLES Refusals.pdf, page 73.

⁹⁹ See clinical encounter titled History of Present Illness, dated December 1, 2020. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt8.pdf, pages 230-231.

¹⁰⁰ See sMARt MAR dated December 2021. File path: Medical record request, Completed medical record DANIEL, CHARLES Complete Medical Record Pt10.pdf, page 319.

¹⁰¹ See Detainee file. File path: General Records Request – NWIPC (SEA), Daniel A022 745 951 Detention File, Daniel 022745951 sec3 pt 1.pdf, pages 15-16.

DATE	CARE REFUSED	SIGNED REFUSAL FORM
2/3/2021	Provider appointment	Refused
2/24/2021	Med: paroxetine	Refused
3/3/2021	Annual PE	Refused
3/15/2021	Annual dental exam	Refused
4/1/2021	COVID vaccine	Refused
4/28/2021	SAAPI mental health evaluation	Refused
5/27/2021	Lab work	Yes
6/1/2021	Provider appointment	Yes
6/2/2021	Chronic care appointment	Yes
6/8/2021	Chronic care appointment	Yes
6/24/2021	Blood pressure	Yes
6/25/2021	Vital signs	Refused
6/25/2021	Vital signs	Refused
8/12/2021	SAAPI interview	Yes
8/25/2021	Lab work	Yes
9/1/2021	Medication: paroxetine	Refused
9/1/2021	Chronic care appointment	Yes
9/8/2021	Medication: paroxetine	Refused
9/9/2021	Medication: paroxetine	Refused
9/10/2021	Medication: paroxetine	Refused
9/11/2021	Medication: paroxetine	Refused
9/12/2021	Medication: paroxetine	Refused
9/28/2021	Vaccines	Yes
10/14/2021	SAAPI complaint evaluation	Yes
10/17/2021	Provider appointment	Yes
11/26/2021	Lab work	Refused
12/1/2021	Provider appointment	Yes

The following table shows DANIEL's recorded blood pressure readings during calendar year 2021.

DATE	BLOOD PRESSURE READING
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DETAINEE DEATH REVIEW: Charles DANIEL, A022745951
Creative Corrections Healthcare and Security Compliance Analysis
May 21, 2024

Page 20

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6/23/2021	165/97
6/24/2021	195/110
6/24/2021	209/116
9/29/2021	120/78

Significant Healthcare Encounters

On January 5, 2021, Dr. (b)(6),(b)(7)(C),(b)(7)(F) assessed DANIEL in the RHU.¹⁰² He noted DANIEL started citalopram per his own request, but stopped taking it on December 15, 2020, because the brand of the medication changed.¹⁰³ Dr. (b)(6),(b)(7)(C),(b)(7)(F) prescribed paroxetine HCl¹⁰⁴, 10 mg, one tablet daily. Dr. (b)(6),(b)(7)(C),(b)(7)(F) informed DANIEL he would not prescribe a new medication in the future when the brand changed. Dr. (b)(6),(b)(7)(C),(b)(7)(F) scheduled a follow-up appointment in one month. Dr. (b)(6),(b)(7)(C),(b)(7)(F) continued the paroxetine during follow up appointments on January 25, 2021¹⁰⁵, February 22, 2021¹⁰⁶, March 31, 2021¹⁰⁷, and April 28, 2021¹⁰⁸, noting DANIEL stated he felt better on the medication.

On February 3, 2021, DANIEL refused to go to a scheduled provider appointment.¹⁰⁹ PA-C (b)(6),(b)(7)(C),(b)(7)(F) spoke to DANIEL in the RHU.¹¹⁰ During ERAU's interview with PA-C (b)(6),(b)(7)(C),(b)(7)(F) he said he felt obliged to check on DANIEL because he had not seen him in a while, commenting also that he believed DANIEL would refuse a blood pressure check and anti-hypertensive medication.¹¹¹ He did not document whether he asked to check his blood pressure or whether he again encouraged DANIEL to accept medication. PA-C (b)(6),(b)(7)(C),(b)(7)(F) observed DANIEL

¹⁰² See clinical encounter titled Refused Citalopram, dated January 5, 2021. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt8.pdf*, pages 136 -137.

¹⁰³ The pharmacy changed the brand of the medication; not the medication itself.

¹⁰⁴ Paroxetine as a medication used for depression.

¹⁰⁵ See clinical encounter titled Delusional Disorder/Secondary depression, dated January 25, 2021. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt8.pdf*, pages 81-82.

¹⁰⁶ See clinical encounter titled History of Present Illness, dated February 22, 2021. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt7.pdf*, pages 500-501.

¹⁰⁷ See clinical encounter titled Delusional Disorder, dated March 31, 2021. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt7.pdf*, pages 391-392.

¹⁰⁸ See clinical encounter titled Delusional Disorder, dated April 28, 2021. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt7.pdf*, pages 306-307.

¹⁰⁹ See ICE Health Service Corps Refusal Form, dated February 3, 2024. File path: *Medical Records Request-NWIPC (SEA), Refusals, DANIEL, CHARLES Refusals.pdf*, pages 65-66.

¹¹⁰ See clinical encounter titled Alleges an officer is threatening to kill him and has been poisoning his food, dated February 3, 2021. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt8.pdf*, page 50.

¹¹¹ ERAU interview with (b)(6),(b)(7)(C),(b)(7)(F) PA, on April 2, 2024.

ambulating in his cell and eating a bowl of food; he smiled and stated he was doing okay.¹¹² DANIEL said he did not feel the need to attend the scheduled provider appointment that day.

On April 28, 2021, during a mental health segregation round, DANIEL told Dr. [REDACTED] that the previous night a staff member made remarks he interpreted as sexual harassment.¹¹³ He said he wanted to make a Sexual Abuse and Assault Prevention and Intervention (SAAPI) complaint. DANIEL refused to go to the medical department for a SAAPI Mental Health Evaluation and answer more questions regarding the incident, stating he already reported the incident and did not want to leave his unit to discuss further.¹¹⁴ [REDACTED], PA, discussed the allegation with DANIEL, during which DANIEL stated he was not physically assaulted in any way.¹¹⁵ He said speech emanates from the vent in his room, and he believed a voice in the vent made the sexual remarks.

On May 27, 2021, Dr. [REDACTED] spoke to DANIEL in the RHU.¹¹⁶ He noted that for the last five months, DANIEL had taken paroxetine which improved daytime tiredness, low energy, and inability to focus. DANIEL reported he felt better taking paroxetine. He again did not accept delusional disorder as a diagnosis and refused medication to treat it. When Dr. [REDACTED] asked DANIEL to accept the COVID-19 vaccine, he replied that his mother died from COVID-19 four days ago but refused to let Dr. [REDACTED] verify his mother's death. DANIEL also stated his sister is a doctor and a vaccination scientist and she told DANIEL to stay away from the vaccine.

On June 23, 2021, during a routine mental health round, [REDACTED], Licensed Clinical Social Worker (LCSW), noted an officer expressed concern about DANIEL's paranoid thoughts and delusions.¹¹⁷ The officer stated DANIEL was banging on his cell door, demanding to speak with a supervisor, and accused GEO of murdering his mother and threatening his brother. LCSW [REDACTED] assessment found DANIEL's thought process was unclear and nonlogical- and he displayed signs of psychosis which was outside his baseline behavior. DANIEL was agitated, his affect was animated, he had poor eye contact, and he demonstrated signs of paranoia. LCSW [REDACTED] discussed DANIEL's behavior at the mental health team meeting the same day and the team determined DANIEL should be admitted to the Medical Housing Unit (MHU) for mental health

¹¹² See clinical encounter titled Alleges an officer is threatening to kill him and has been poisoning his food, dated February 3, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt8.pdf, page 50.*

¹¹³ Pages 308 as well titled SAAPI314 and 315 part 7 of complete medical record. See MH seg rounds dated 4/28/2021.

¹¹⁴ See ICE Health Service Corps Refusal Form, dated April 28, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf, page 288.*

¹¹⁵ See clinical encounter titled SAAPI, dated April 28, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt7.pdf, pages 312-313.*

¹¹⁶ See clinical encounter titled Delusional disorder/Adjustment disorder, dated May 27, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt7.pdf, page 224-225.*

¹¹⁷ See clinical encounter titled Mental health seg, dated June 23, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt7.pdf, pages 159-161.*

observation. LCSW [REDACTED] scheduled follow-up appointments with Dr. [REDACTED] and Dr. [REDACTED].

The same day, Dr. [REDACTED] assessed DANIEL for admission to the MHU for mental health observation.¹¹⁸ She noted the following:

- DANIEL was pleasant and cooperative and denied suicidal thoughts or audio or visual hallucinations.
- DANIEL said no one believes him and that electronic waves are monitoring his thinking.
- DANIEL reported depressed mood and feeling distressed.
- DANIEL reported his mother died one month ago.
- DANIEL denied sleep problems.
- DANIEL appeared to have increased paranoid thoughts and delusions and recent delusional thoughts about GEO officers.
- DANIEL was at low risk for suicide.

Dr. [REDACTED] admitted DANIEL to the MHU for mental health observation.¹¹⁹ She ordered 15-minute irregular checks by custody staff and nursing checks every eight hours, and no restrictions on items DANIEL was allowed to have in his cell. She also ordered vital signs and provider checks daily. She noted DANIEL would be discharged from MHU when he demonstrated a continuous lack of suicide thoughts, prosocial problem solving, and stability of mood and safe behaviors.

[REDACTED], RN, completed the requisite MHU RN admission encounter, noting DANIEL's vital signs were within normal limits except for elevated blood pressure of 165/97.¹²⁰ DANIEL denied symptoms associated with high blood pressure. RN [REDACTED] notified PA-C [REDACTED] of the elevated blood pressure reading and scheduled an appointment with him the next day.

On June 24 and 25, 2021, Dr. [REDACTED] Ph.D., assessed DANIEL.¹²¹ On June 25, 2021, DANIEL displayed safe behaviors, reported future focus, a sense of resiliency and a belief system that discouraged suicide.¹²² She discussed DANIEL's condition with the mental health

¹¹⁸ See clinical encounter titled MH SRA, dated June 23, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt7.pdf, pages 151-154.*

¹¹⁹ See clinical encounter titled MHU Admit, dated June 23, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt7.pdf, pages 155-156.*

¹²⁰ See clinical encounter titled MHU RN ADMISSION, dated June 23, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt7.pdf, pages 148-149.*

¹²¹ See clinical encounter titled MH SRA, dated June 24, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt7.pdf, pages 140-142;* and clinical encounter titled MH SRA, dated June 25, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt7.pdf, pages 125-127.*

¹²² See clinical encounter titled ADM DISCHARGE FROM MHU, dated June 25, 2021. *File path: Medical Records Request-NWIPC (SEA), DANIEL, CHARLES Complete Medical Record Pt7.pdf, pages 117-118.*

treatment team which concluded DANIEL could safely return to the RHU.¹²³ He was discharged from the MHU at 1:00 p.m. and returned to the RHU.¹²⁴

On July 19, 2021, Dr. [REDACTED] assessed DANIEL in the RHU.¹²⁵ He noted DANIEL's delusional thoughts had not changed in intensity. Dr. [REDACTED] continued DANIEL's paroxetine prescription and noted he wanted to explore the possibility of placement in another ICE facility that could force treatment for psychosis. He also noted DANIEL's symptoms were not presenting an acute danger to self or others since he was in a single cell. During an interview with ERAU, Dr. [REDACTED] stated that he was aware that action was taken at some point pursuant to his recommendation for DANIEL's transfer, but he was not sure by whom.¹²⁶

On August 11, 2021, PA-C [REDACTED] spoke with DANIEL in his RHU cell regarding a SA-API allegation.¹²⁷ DANIEL stated an officer made a sexually explicit comment and boasted about murdering DANIEL's mother. DANIEL stated there was no exposure to bodily fluids and no physical contact. The next day, August 12, 2021, Dr. [REDACTED] interviewed DANIEL regarding his SA-API allegation.¹²⁸ DANIEL refused to be interviewed in Dr. [REDACTED] office because he did not want to leave his cell, stating he could not leave his food and documents. Dr. [REDACTED] spoke with DANIEL through the cell's cuff port and DANIEL repeated his allegation about an officer harassing him. Dr. [REDACTED] discussed safety and reporting with DANIEL. During an interview with ERAU [REDACTED], Administrative Lieutenant and PREA Compliance Manager, he stated all SA-API investigations he investigated pertaining to DANIEL were unfounded.¹²⁹

On September 1, 8, 9, and 10, 2021, DANIEL refused his paroxetine medication.¹³⁰ [REDACTED] Behavioral Health Therapist (BHT), met with DANIEL on September 10, 2021, regarding his medication refusal.¹³¹ DANIEL stated the medication was not working since the brand changed. DANIEL reiterated his concern about the brand change to PA-C [REDACTED] the same day, stating the government can modify the tablets using sound waves coming through the

¹²³ See clinical encounter titled MH SRA, dated June 25, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt7.pdf, pages 125-127.*

¹²⁴ See clinical encounter titled ADM DISCHARGE FROM MHU, dated June 25, 2021. *File path: Medical Records Request-NWIPC (SEA), DANIEL, CHARLES Complete Medical Record Pt7.pdf, pages 117-118.*

¹²⁵ See clinical encounter titled Severe Delusions, dated July 19, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt7.pdf, pages 48-49.*

¹²⁶ ERAU interview with Dr. Mallya on April 3, 2024.

¹²⁷ See clinical encounter titled SA-API, dated August 11, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt6.pdf, pages 484-485.*

¹²⁸ See encounter titled SA-API, dated August 12, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt60.pdf, page 478-481.*

¹²⁹ ERAU interview with [REDACTED], Lieutenant, on April 3, 2024.

¹³⁰ See ICE Health Service Corps Refusal Forms, dated September 1, 8, 9, 10, 2021. *File path: Medical Records Request-NWIPC (SEA), Refusals, DANIEL, CHARLES Refusals.pdf, pages 35, 40, 42 and 44.*

¹³¹ See encounter titled Wellness check for medication refusal, dated September 10, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt6.pdf, page 399.*

vents in his cell.¹³² DANIEL refused the medication again on September 11 and 12, 2021¹³³, and on September 13, 2021, Dr. [REDACTED] discontinued the paroxetine prescription.¹³⁴

On October 13, 2021, during mental health RHU rounds, DANIEL told Dr. [REDACTED] he would like to try a new antidepressant medication.¹³⁵ The same day, DANIEL submitted a grievance in which he asked that Dr. [REDACTED] provide choices of medications until he finds one that works.¹³⁶ After discussing the request with Dr. [REDACTED] Dr. [REDACTED] informed DANIEL that Dr. [REDACTED] would not change the medication again because of DANIEL's concern about the brand. Dr. [REDACTED] informed DANIEL that Dr. [REDACTED] was willing to prescribe an antipsychotic if DANIEL was willing to take it.¹³⁷ On the response to the grievance, Dr. [REDACTED] wrote that changing medication as requested, was not clinically justified.¹³⁸

On October 23, 2021, DANIEL submitted a sick call request asking to resume antidepressant medication.¹³⁹ Dr. [REDACTED] response was that ordering medications that were not helpful in treating his delusions would exacerbate them, and that he would not prescribe an antidepressant.¹⁴⁰ Dr. [REDACTED] previously communicated this information to DANIEL as well as the mental health treatment team. Dr. [REDACTED] recorded he would not meet with DANIEL unless he showed a willingness to start antipsychotic medications.

January 1, 2022 - December 31, 2022

Security Summary

¹³² See encounter titled Medication refusal, dated September 10, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt6.pdf, page 397.*

¹³³ See ICE Health Service Corps Refusal Forms, dated September 11 and 12, 2021. *File path: Medical Records Request-NWIPC (SEA), Refusals, DANIEL CHARLES Refusals.pdf, pages 37 and 39.*

¹³⁴ See sMARt MAR, dated September 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf, page 218.*

¹³⁵ See encounter titled Mental health, seg dated October 13, 2021. *File path: Medical record request, Completed medical record DANIEL, CHARLES Complete Medical Record Pt6.pdf, page 300-301.*

¹³⁶ See Northwest ICE Processing center Detainee Informal Grievance Form, dated October 13, 2021. *File path: Medical Records Request-NWIPC (SEA), Sick Call and Grievances, CHARLES DANIELS Sick Calls.Grievances.pdf, pages 15-16.*

¹³⁷ See encounter titled Mental health, seg dated October 13, 2021. *File path: Medical record request, Completed medical record DANIEL, CHARLES Complete Medical Record Pt6.pdf, page 300-301.*

¹³⁸ See Northwest ICE Processing center Detainee Informal Grievance Form, dated October 13, 2021. *File path: Medical Records Request-NWIPC (SEA), Sick Call and Grievances, CHARLES DANIELS Sick Calls.Grievances.pdf, pages 15-16.*

¹³⁹ See ICE Health Service Corps Sick Call Request Form, dated October 22, 2021. *File path: Medical Records Request-NWIPC (SEA), Sick Call and Grievances, CHARLES DANIEL Sick Calls.Grievances.pdf, page 8.*

¹⁴⁰ See encounter titled Delusional and requesting to resume Paroxetine, dated October 23, 2021. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt6.pdf, pages 268-269.*

DANIEL remained on protective custody. Security personnel conducted required status reviews during which DANIEL requested continuation on protective custody.

Healthcare Summary

Nurses and mental health professionals documented required rounds. DANIEL continued to refuse offered care, including blood draw for lab tests and hypertension chronic care clinic, vital signs, and medical and mental health provider appointments.

DATE	CARE REFUSED	SIGNED REFUSAL FORM
2/25/2022	Dental appointment	Yes
2/25/2022	Lab work	Refused
3/2/2022	Provider appointment	Yes
3/3/2022	Sick call appointment	Refused
3/3/2022	Provider appointment	Yes
5/25/2022	Lab work	Refused
8/11/2022	Lab work	Refused
8/26/2022	COVID swab	Refused
9/14/2022	COVID lab	Refused
9/14/2022	Ear Nose and Throat (ENT) consult	Yes
10/6/2022	Assessment of cyst at hospital ENT clinic	Yes
11/15/2022	Blood work	Refused
11/20/2022	Blood pressure check	Yes
11/21/2022	Provider appointment	Yes
12/4/2022	Vital signs appointment	Refused

In 2022, six blood pressure readings were recorded, all of which were elevated.

DATE	BLOOD PRESSURE READING
3/3/2022	186/112
8/10/2022	163/99
8/11/2022	154/82; 150/70
8/17/2022	168/90
8/25/2022	162/86
8/26/2022	148/101

Significant Healthcare Encounters

On March 3, 2022, DANIEL refused to go to the medical clinic for an appointment but permitted PA-C (b)(6),(b)(7)(C),(b)(7)(F) to obtain vital signs in the RHU.¹⁴¹ DANIEL's blood pressure was elevated at 186/112; other vital signs were normal. During an interview with ERAU, PA-C (b)(6),(b)(7)(C),(b)(7)(F) stated he did not address the blood pressure reading because DANIEL consistently refused treatment for hypertension.¹⁴² PA-C (b)(6),(b)(7)(C),(b)(7)(F) also said he did not treat DANIEL's hypertension as emergent because he did not have related symptoms.¹⁴³ He acknowledged attempts to check DANIEL's blood pressure ceased going forward because of DANIEL's pattern of refusals.

On May 4, 2022, DANIEL submitted a sick call request again asking to resume antidepressant medication.¹⁴⁴ Dr. (b)(6),(b)(7)(C),(b)(7)(F) declined DANIEL's request because DANIEL would not acknowledge his mental illness.¹⁴⁵

On May 18, 2022, Dr. (b)(6),(b)(7)(C),(b)(7)(F) saw DANIEL in the RHU regarding a grievance sent to Dr. (b)(6),(b)(7)(C),(b)(7)(F), regarding denial of antidepressant medication.¹⁴⁶ Dr. (b)(6),(b)(7)(C),(b)(7)(F) ordered paroxetine, 10 mg once daily as needed.¹⁴⁷ DANIEL signed a consent form for the medication.¹⁴⁸

On July 2, 2022, PA-C (b)(6),(b)(7)(C),(b)(7)(F) discontinued DANIEL's paroxetine prescription.¹⁴⁹ According to the MAR, since Dr. (b)(6),(b)(7)(C),(b)(7)(F) restarted the medication on May 18, 2022, DANIEL accepted paroxetine doses May 18 - 22, 2022, and on June 9, 2022.¹⁵⁰

¹⁴¹ See encounter titled Vitals, dated March 3, 2022. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt5.pdf, page 421.

¹⁴² ERAU interview with PA-C (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024.

¹⁴³ PA-C (b)(6),(b)(7)(C),(b)(7)(F) provided ERAU with citations to research that indicates hypertensive urgency, in the absence of target end organ damage, has not been linked to increased morbidity or mortality. See Warnstadt, Emily, email, dated April 3, 2024. File path: Medical Records Request-NWIPC (SEA), Email, FW_OPR Interview4_2_24 - Daniel.msg, page 1.

¹⁴⁴ See ICE Health Service Corps Sick Call Request Form, dated May 4, 2022. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf, page 178.

¹⁴⁵ See encounter titled MH Sick Call, dated May 5, 2022. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt5.pdf, pages 256-258 421.

¹⁴⁶ See encounter titled Pt seen for AC: addressed grievance, dated May 18, 2022. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt5.pdf, page 219-220.

¹⁴⁷ See sMARt MAR, dated May 2022. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf, page 168.

¹⁴⁸ See CONSENT TO USE OF SEROTONIN REUPTAKE INHIBITOR ANTIDEPRESSANT MEDICATION form, dated May 18, 2022. File path: Medical Records Request-NWIPC (SEA), Treatment Authorization, DANIEL_CHARLES_Treatment Authorization Requests.pdf, page 1.

¹⁴⁹ See sMARt MAR, dated July 2022. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf, page 158.

¹⁵⁰ See sMARt MAR, dated June 2022. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf, pages 164 and 168.

On August 10, 2022, PA-C (b)(6),(b)(7)(C),(b)(7)(F) evaluated DANIEL in the medical clinic.¹⁵¹ Preceding the encounter, on August 8, 2022, DANIEL submitted a sick call request for an ankle wrap, stating he did not want to have a visit for this request.¹⁵² PA-C (b)(6),(b)(7)(C),(b)(7)(F) spoke with DANIEL in the RHU the next day, noting the request for a wrap was due to chronic ankle swelling.¹⁵³ He informed DANIEL he would have to be seen in the medical clinic, and DANIEL agreed. During the encounter on August 10, 2022, PA-C (b)(6),(b)(7)(C),(b)(7)(F) noted the following:¹⁵⁴

- DANIEL's vital signs were normal except for elevated blood pressure of 163/99.
- DANIEL reported swelling in his left ankle and foot for one month with no related history of trauma or injury. He denied left leg pain and tingling, but stated in the night when he is attacked by electronic waves, he wrapped the knee and the pain would go away. PA-C (b)(6),(b)(7)(C),(b)(7)(F) noted the entire left leg was mildly larger compared to his right leg. The leg had full range of motion.
- DANIEL reported decreased hearing in his right ear. PA-C (b)(6),(b)(7)(C),(b)(7)(F) noted the tympanic membrane in the ear was perforated.¹⁵⁵
- DANIEL had a neck mass which he said he first noticed as a small lump in September of 2021. He denied pain in his neck, fever, dyspnea,¹⁵⁶ or difficulty swallowing. PA-C (b)(6),(b)(7)(C),(b)(7)(F) note the mass was firm and protruding two by 2.5 inches, anterior to the larynx and trachea, and it was painless.

Based on his assessment, PA-C (b)(6),(b)(7)(C),(b)(7)(F) recommended medications to treat DANIEL's hypertension, which DANIEL refused. PA-C (b)(6),(b)(7)(C),(b)(7)(F) also submitted a referral for an ultrasound¹⁵⁷ of the neck mass and ordered a urinalysis dipstick¹⁵⁸, comprehensive metabolic panel (CMP)¹⁵⁹, and complete blood count (CBC) with differential/platelet¹⁶⁰.

¹⁵¹ See encounter titled LE edema, dated August 10, 2022. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt4.pdf, pages 486-488.

¹⁵² See ICE Health Service Corp Sick Call Request Form, dated August 8, 2022. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf, page 153.

¹⁵³ See encounter titled Requesting ankle wraps?, dated August 9, 2022. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt4.pdf, pages 494-495.

¹⁵⁴ See encounter titled LE edema, dated August 10, 2022. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt4.pdf, pages 486-488.

¹⁵⁵ Tympanic membrane, also known as eardrum, is a thin, circular layer of tissue that separates the outer ear from the middle ear.

¹⁵⁶ Dyspnea is shortness of breath.

¹⁵⁷ An ultrasound is a noninvasive procedure that uses high-frequency sound waves to produce images of structures within the body.

¹⁵⁸ Urinalysis dipstick is a test of urine using a special strip of paper that is dipped into a sample of urine. The result is available almost immediately. Also called rapid urine test.

¹⁵⁹ CMP is a group of 14 tests that measures substances in the blood to determine fluid balance, electrolyte levels, and level of functioning of the kidneys and liver.

¹⁶⁰ CBC with differential and platelets evaluates the overall health of the body and detects a wide range of disorders, including anemia, infection, inflammation, bleeding disorders, or cancer. Platelets are a type of blood cell that help clot the blood.

On August 11, 2022, Dr. [REDACTED] evaluated DANIEL's neck mass.¹⁶¹ DANIEL said he treated the mass by gargling with soap and salt water. DANIEL refused to provide a blood sample for the ordered lab tests and stated outside forces were draining people. He agreed to an X-ray and ultrasound of the neck mass. DANIEL's vital signs were normal except for elevated blood pressure readings of 154/82 and 150/70.

On August 14, 2022, DANIEL refused a scheduled ear/nose and throat (ENT) consultation with a specialist.¹⁶² On the refusal form, DANIEL wrote he refused the consultation due to what he called a premature surgery misunderstanding.

On August 15, 2022, PA-C [REDACTED] evaluated DANIEL and noted the neck mass was larger.¹⁶³ DANIEL complained about increased swelling in his right leg above the knee. PA-C [REDACTED] noted pitting edema¹⁶⁴ in both extremities, tenderness in the back of the left knee with no mass felt, and increased fluid above the left kneecap with no gross swelling in the thigh. Based on PA-C [REDACTED] findings, he ordered the following labs: prostate-specific antigen (PSA);¹⁶⁵ CMP; thyroid stimulating hormone (TSH);¹⁶⁶ and tests to detect sexually transmitted diseases. He also ordered an ACE™ bandage and knee sleeve for edema in DANIEL's left leg¹⁶⁷, and requested an ultrasound of the neck mass.¹⁶⁸ The ultrasound, approved and completed the same day, identified midline neck fluid collection and included a recommendation to have a contrast-enhanced computed tomography (CT)¹⁶⁹ of the neck.¹⁷⁰

¹⁶¹ See encounter titled Neck Mass, dated August 11, 2022. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt4.pdf, pages 481-482.*

¹⁶² See ICE Health Service Corps Refusal Form, dated August 11, 2022. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf, page 128.*

¹⁶³ See encounter titled Multiple complaints, dated August 15, 2022. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt4.pdf, pages 470-471.*

¹⁶⁴ Excess fluid build up in the body, causing swelling.

¹⁶⁵ PSA Total is a blood test used to screen for prostate cancer.

¹⁶⁶ A thyroid stimulating hormone (TSH) test measures the amount of TSH in the blood. TSH is produced by the pituitary gland.

¹⁶⁷ See encounter Special Needs Approved, dated August 15, 2022. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt4.pdf, pages 468.*

¹⁶⁸ See REFERRAL and Consultation Request and hospital Transfer form, dated August 15, 2022. *File path: Medical Records Request-NWIPC (SEA), Provider Orders, CHARLES DANIELS Provider Orders.pdf page 1.*

¹⁶⁹ A contrast-enhanced CT refers to a technique used during a CT scan whereby a contrast agent is administered to the patient which allows certain structures in the body to become more clearly visible on CT images.

¹⁷⁰ See TridentCare DIAGNOSTIC LABORATORIES report, dated August 15, 2022. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf, page 149.*

On August 16, 2022, Dr. (b)(6),(b)(7)(C),(b)(7)(F) requested a doppler ultrasound¹⁷¹ of the left leg¹⁷² to look for a deep vein thrombosis (DVT)¹⁷³ of popliteal¹⁷⁴ and thigh. The ultrasound, approved and completed the same day, identified no evidence of DVT.¹⁷⁵

Also on this date, Dr. (b)(6),(b)(7)(C),(b)(7)(F) sent an email to (b)(6),(b)(7)(C),(b)(7)(F) LCDR, Health Services Administrator (HSA), following up on Dr. (b)(6),(b)(7)(C),(b)(7)(F) July 2021 recommendation to explore the possibility DANIEL's transfer to another ICE facility.¹⁷⁶ HSA (b)(6),(b)(7)(C),(b)(7)(F) forwarded Dr. (b)(6),(b)(7)(C),(b)(7)(F) email to (b)(6),(b)(7)(C),(b)(7)(F).¹⁷⁷ During an interview with ERAU, HSA (b)(6),(b)(7)(C),(b)(7)(F) stated later efforts to seek DANIEL's transfer (see below) were handled mainly by TIPC's Clinical Director and the Assistant Field Officer Director.¹⁷⁸

On August 17, 2022, Dr. (b)(6),(b)(7)(C),(b)(7)(F) conducted a follow-up encounter regarding DANIEL's neck mass.¹⁷⁹ DANIEL's vital signs were normal except for elevated blood pressure of 168/90. During an interview, ERAU asked (b)(6),(b)(7)(C),(b)(7)(F) about DANIEL's consistently elevated blood pressure readings.¹⁸⁰ He acknowledged the readings were high, but noted DANIEL did not have symptoms associated with high blood pressure.

Dr. (b)(6),(b)(7)(C),(b)(7)(F) reviewed the ultrasound reports with DANIEL, noting the ultrasound of the left leg was negative and showed no evidence of DVT; also, that the ultrasound of the neck identified a thyroglossal duct cyst¹⁸¹. During the encounter DANIEL complained of draining in the right ear with intermittent tinnitus¹⁸². He denied pain, fever or chills, shortness of breath, or cough. Dr. (b)(6),(b)(7)(C),(b)(7)(F) noted DANIEL's right ear was blocked with clear discharge.

Dr. (b)(6),(b)(7)(C),(b)(7)(F) diagnoses were infected otitis media¹⁸³ of the right ear, presumptive thyroglossal cyst and uncontrolled blood pressure. He prescribed neomycin-polymyxin-hydrocortisone (HC), four drops twice a day for seven days. Dr. (b)(6),(b)(7)(C),(b)(7)(F) ordered a prothrombin test (PT) and partial

¹⁷¹ A doppler ultrasound is a noninvasive procedure that uses high-frequency sound waves to measure the amount of blood flow through arteries and veins. It is often used to detect abnormal flow within an artery or blood vessel.

¹⁷² See REFERRAL and Consultation Request and Hospital Transfer form, dated August 16, 2022. File path: *Medical Records Request-NWIPC (SEA), Provider Orders, CHARLES DANIELS Provider Orders.pdf* page 2.

¹⁷³ DVT is a blood clot in a deep vein, most commonly in the legs or pelvis.

¹⁷⁴ Popliteal is relating to or situated in the back of the knee.

¹⁷⁵ See TridentCare DIAGNOSTIC LABORATORIES report, dated August 16, 2022. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf*, page 148.

¹⁷⁶ See Malakhova, Sheri email, dated August 16, 2022. File path: *Medical Records Request-NWIPC (SEA), Emails, FW RE A#022745951 60 year old from Trinidad.msg*, page 1.

¹⁷⁷ (b)(6),(b)(7)(C),(b)(7)(F) has an ICE email address; however, Creative Corrections does not know his title or officer. A reply from him to HSA (b)(6),(b)(7)(C),(b)(7)(F) was not in the email chain provided.

¹⁷⁸ ERAU interview with HSA (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024.

¹⁷⁹ See encounter titled Neck Mass f/u, dated August 17, 2022. File path: *Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt14.pdf*, pages 458-459.

¹⁸⁰

¹⁸¹ Thyroglossal duct cysts are rare cysts in the throat that are present at birth and almost always benign.

¹⁸² Tinnitus is ringing in the ear(s).

¹⁸³ Infected otitis media is a bacterial infection of the middle ear.

prothrombin test (PTT)¹⁸⁴ labs and follow-up in one week. DANIEL was cooperative with a blood draw for lab work ordered by PA-C (b)(6),(b)(7)(C),(b)(7)(F) and Dr. (b)(6),(b)(7)(C),(b)(7)(F).¹⁸⁵

On August 25, 2022, PA-C (b)(6),(b)(7)(C),(b)(7)(F) met with DANIEL to review lab results.¹⁸⁶ DANIEL's vital signs were normal except for elevated blood pressure of 162/86. The lab results were normal except for the following:¹⁸⁷

- Slightly elevated creatinine of 1.41¹⁸⁸.
- Elevated prolactin 18.2 ng/mL¹⁸⁹.
- High PSA of 4.4¹⁹⁰.
- Low estimated Glomerular Filtration Rate (eGFR) of 57.¹⁹¹

PA-C (b)(6),(b)(7)(C),(b)(7)(F) discussed the lab results with DANIEL, including further screening for prostate cancer. DANIEL denied associated urinary symptoms, abdominal, groin or pelvic pain. Regarding the neck mass, DANIEL reported no new or worsening symptoms.

PA-C (b)(6),(b)(7)(C),(b)(7)(F) discussed DANIEL's lab results and the neck mass with Dr. (b)(6),(b)(7)(C),(b)(7)(F) and noted a CT with contrast would be scheduled¹⁹². He encouraged DANIEL to take medication for chronic hypertension and discussed the risk factors of uncontrolled hypertension, including low kidney function as indicated by lab results.

On August 26, 2022, a CT was completed at St. Joseph Medical Center.¹⁹³ DANIEL refused the intravenous fluid required for the contrast-enhanced CT so a CT without contrast was

¹⁸⁴ PT and PTT measure how quickly blood clots.

¹⁸⁵ See encounter titled Prolactin, oot, cbc, comp, thyroid, pt/ptt, etc, dated August 17, 2022. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt4.pdf, pages 457-458.

¹⁸⁶ See encounter titled Lab review/Opt Out Testing Results, dated August 25, 2022. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt4.pdf, pages 430-432.

¹⁸⁷ See lab results, dated August 19, 2022. File path: Medical Records Request-NWIPC (SEA), Labs and X-Ray, CHARLES DANIELS Lab.X-Ray Results.pdf, pages 2-16.

¹⁸⁸ A creatinine test is a measure of how well the kidneys are functioning. Normal creatine is 0.76-1.27 mh.

¹⁸⁹ A prolactin test is a measure of how much hormone prolactin is found in the blood. Normal prolactin is 4.0-15.2 ng/mL.

¹⁹⁰ Normal PSA is 0.0-4.0 ng/mL.

¹⁹¹ An eGFR test shows how well the kidneys are filtering. Normal eGFR is >59 (ml.min.1.73).

¹⁹² PA-C Robinson ordered an urgent referral for contrast-enhanced CT of the neck on August 26, 2022. The referral was approved the same day. See REFERRAL and Consultation Request and hospital Transfer form, dated August 16, 2022. File path: Medical Records Request-NWIPC (SEA), Provider Orders, CHARLES DANIELS Provider Orders.pdf, page 3.

¹⁹³ See encounter titled Radiology appointment at St. Joseph Outpatient on 8/26/2022 at 1430 for CT neck English Speaking, dated August 26, 2022. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt4.pdf, pages 426-427.

completed.¹⁹⁴ The CT identified a smooth cystic lesion¹⁹⁵ to the left of midline of the anterior¹⁹⁶ neck, measuring approximately six centimeters (cm) in width.

On September 1, 2022, PA-C [REDACTED] met with DANIEL to discuss findings of the CT scan.¹⁹⁷ The examination findings were normal except for localized swelling of the neck mass and chronic infection of the right ear. PA-C [REDACTED] prescribed ofloxacin 0.3 percent¹⁹⁸, five drops three times a day for 14 days. He discussed surgical consultation to remove the neck mass. DANIEL declined, stating he wanted to complete the antibiotic treatment because he believed the ear infection and neck mass were related and caused by electronic wave torture and attacking by the government. PA-C [REDACTED] agreed to revisit the surgical consult after the antibiotic treatment for the ear infection.

On September 14, 2022, Dr. [REDACTED] submitted a referral for surgical consultation with an otolaryngology¹⁹⁹ specialist for DANIEL's neck mass.²⁰⁰ DANIEL refused the consultation on grounds he believed it was a premature surgery misunderstanding.²⁰¹

On September 22, 2022, PA-C [REDACTED] conducted a follow-up appointment to assess DANIEL's ear infection.²⁰² DANIEL had finished the course of antibiotics and denied ear pain. DANIEL reported improved hearing and PA-C [REDACTED] recommended continued monitoring.

On October 3, 2022, PA-C [REDACTED] again requested surgical consultation for the mass on DANIEL's neck.²⁰³ During an interview with ERAU, PA-C [REDACTED] commented DANIEL refused consultation twice before.²⁰⁴ As noted above, during his encounter with DANIEL on September 1, 2022, DANIEL said he wanted to complete the course of antibiotics before agreeing to surgical consultation on the mass because he believed the ear infection and neck mass were related. During PA-C [REDACTED] last encounter with DANIEL on September 22,

¹⁹⁴ See St. Joseph Medical Center Imaging Result, dated August 30, 2022. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf, pages 136-137.*

¹⁹⁵ A smooth cystic lesion is a sac that may be filled with air, fluid or other material.

¹⁹⁶ Anterior means toward the front.

¹⁹⁷ See encounter titled CT results, dated September 1, 2022. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt4.pdf, pages 409-410.*

¹⁹⁸ Ofloxacin is an antibiotic that treats infections caused by bacteria. It is used to treat outer ear infections.

¹⁹⁹ Otolaryngology is a medical specialty for the study of ear, nose and throat.

²⁰⁰ See U.S. Immigration and Customs Enforcement REFERRAL Consultation Request and Hospital Transfer Form, dated September 14, 2022. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt10.pdf, pages 493.*

²⁰¹ See IHSC Refusal Form, dated September 14, 2022. *File path: Medical Records Request-NWIPC (SEA), Refusals, DANIEL CHARLES Refusals.pdf page 23.*

²⁰² See encounter titled F/u ear infection, dated September 22, 2022. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt4.pdf, pages 349-350.*

²⁰³ See REFERRAL and Consultation Request and hospital Transfer form, dated August 16, 2022. *File path: Medical Records Request-NWIPC (SEA), Provider Orders, CHARLES DANIELs Provider Orders.pdf page 4.*

²⁰⁴ ERAU interview with PA-C [REDACTED], PA on April 2, 2024.

2024, PA-C (b)(6),(b)(7)(C),(b)(7)(F) determined his hearing was improved following completion of the course of antibiotics.

On October 6, 2022, DANIEL again refused surgical consult for removal of the neck mass.²⁰⁵ DANIEL told PA-C (b)(6),(b)(7)(C),(b)(7)(F) he refused the surgery because he believed that upon his return, he would be subject to electronic bombardment and attacks that would penetrate through his surgical wound.²⁰⁶ DANIEL refused a blood pressure check but allowed other vital signs checks, which were normal. PA-C (b)(6),(b)(7)(C),(b)(7)(F) also documented GEO officers were concerned DANIEL was losing weight, but PA-C (b)(6),(b)(7)(C),(b)(7)(F) noted DANIEL had gained weight over the past two to three months. DANIEL refused to wear compression stockings and asked for replacement ACE™ wraps. PA-C (b)(6),(b)(7)(C),(b)(7)(F) encouraged DANIEL to attend the otolaryngology appointment, elevate his legs above his heart for 30 minutes three times a day and increase exercise.

January 1, 2023 - December 31, 2023

Security Summary

DANIEL remained on protective custody. Security personnel conducted required status reviews during which he requested continuation on protection custody. Documentation of weekly multidisciplinary committee meetings from October through December, confirmed his status was discussed at the meetings.²⁰⁷

Healthcare Summary

Nurses and mental health professionals documented required rounds. As discussed below, on August 15, 2023, mental health staff designated DANIEL as a detainee with serious mental illness (SMI), requiring daily mental health rounds per IHSC policy and placement of a global alert. His medical record supports compliance with IHSC requirements once he was designated as meeting SMI criteria.

DANIEL often refused offered care, including annual physical examination, blood draw for lab tests, hypertension chronic care clinic, and provider appointments. No blood pressure readings were recorded in 2023.

²⁰⁵ See encounter titled refuse ENT Consult, dated October 6, 2022. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt4.pdf, pages 312-313.*

²⁰⁶ See encounter titled refuse ENT Consult, dated October 6, 2022. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt4.pdf, pages 312-313.*

²⁰⁷ See Special Management Unit Weekly Reviews, *File Path: General Records, RHU 6 months of weekly multidisciplinary committee reviews.*

DATE	CARE REFUSED	SIGNED REFUSAL FORM
2/10/2023	Blood draw for lab work	Refused
2/15/2023	Chronic care appointment	Yes
3/1/2023	Provider appointment	Yes
3/14/2023	Annual PE	Refused
3/30/2023	Provider appointment	Yes
5/3/2023	Blood draw	Refused
5/12/2023	Chronic care appointment	Yes
8/3/2023	3-month hypertension follow up	Yes
8/30/2023	Vaccines	Refused
10/25/23	Blood draw for lab work	Refused
2/1/2024	Provider appointment	Yes

Significant Healthcare Encounters

On March 29, 2023, (b)(6),(b)(7)(C),(b)(7)(F) RN, evaluated DANIEL in the RHU in response to a request from a mental health social worker to assess DANIEL for swelling in his left lower leg.²⁰⁸ DANIEL was ambulating in his cell without difficulty, and he denied pain. DANIEL jumped up and down in his cell to demonstrate he was not in pain. RN (b)(6),(b)(7)(C),(b)(7)(F) noted edema in his legs, but DANIEL refused a physical assessment. RN (b)(6),(b)(7)(C),(b)(7)(F) scheduled an appointment with a provider the next day, which DANIEL refused.²⁰⁹

On August 15, 2023, Dr. (b)(6),(b)(7)(C),(b)(7)(F) changed DANIEL's status to a detainee with serious mental illness (SMI) after discussion with IHSC headquarters personnel.²¹⁰ During an interview with Dr. (b)(6),(b)(7)(C),(b)(7)(F) she stated she completed the required form (IHSC Form 833)²¹¹, placed a global alert²¹², and instituted the requirement for daily checks by mental health staff per IHSC policy²¹³. DANIEL's medical record documents compliance with the IHSC requirement for mental health staff to complete daily rounds.

²⁰⁸ See encounter titled LE edema, dated March 29, 2023. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt3.pdf, pages 363-364.*

²⁰⁹ See IHSC Refusal Form, dated March 30, 2022. *File path: Medical Records Request-NWIPC (SEA), Refusals, DANIEL CHARLES Refusals.pdf, page 9.*

²¹⁰ ERAU interview with Dr. (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024.

²¹¹ The completed IHSC Form 833 was not provided to ERAU.

²¹² A global alert ensures providers who access a patient's medical record are aware the patient has been diagnosed with an SMI.

²¹³ See IHSC Directive: 07-05, Serious Mental Disorders and Conditions, dated July 25, 2019. *File path: Medical Records Request-NWIPC (SEA), Policies, Serious Mental Disorders and Conditions.pdf, page 3.*

On August 19, 2023, during a daily SMI check (b)(6),(b)(7)(C),(b)(7)(F), LCSW, noted DANIEL was exhibiting symptoms of serious mental illness in the form of expressing delusions about his food.²¹⁴

On October 23, 2023, during a mental health round, Dr. (b)(6),(b)(7)(C),(b)(7)(F) noted DANIEL was well groomed and had good attention. When asked how he was, DANIEL stated he was coming along. Dr. (b)(6),(b)(7)(C),(b)(7)(F) noted DANIEL had not left his room for showers or recreation. She noticed a strong odor emanating from the cell and encouraged him to take a shower. DANIEL declined and stated he has a history of dermatitis that flares up when he showers. He reported he cleans himself using the sink in his room and he felt much cleaner, and the dermatitis had not returned.

January 1, 2024 - Date of Death, March 7, 2024

Security Summary

DANIEL remained on protective custody. Security personnel conducted required status reviews and staff during which DANIEL requested continued protective custody.

Healthcare Summary

Nurses and mental health professionals documented required rounds. DANIEL refused offered care, including annual physical examination and one provider appointment. No blood pressure readings were recorded in 2024.

DATE	CARE REFUSED	SIGNED REFUSAL FORM
2/1/2024	Provider appointment	Yes
3/2/2024	Annual PE	Yes
3/5/2024	Annual dental exam	Yes

Email correspondence provided by HSA (b)(6),(b)(7)(C),(b)(7)(F) reflects that starting on January 5, 2024, Dr. (b)(6),(b)(7)(C),(b)(7)(F) and Dr. (b)(6),(b)(7)(C),(b)(7)(F) pursued DANIEL's transfer to the Krome North Service Processing Center (KNSPC).²¹⁵ On January 10, 2024, Dr. (b)(6),(b)(7)(C),(b)(7)(F) informed Dr. (b)(6),(b)(7)(C),(b)(7)(F) that after discussion, he and the IHSC Western Regional Clinical Director determined there was no medical basis for

²¹⁴ See encounter titled daily SMI check, dated August 19, 2023. File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt2.pdf, pages 489-490.

²¹⁵ See emails dated January 5 – 16, 2024. File path: Medical Records Request-NWIPC (SEA), Emails.

transferring DANIEL to KNSPC and administrative transfer would need to be pursued.²¹⁶ In an email to Dr. [REDACTED] on January 16, 2024, [REDACTED] KNSPC Clinical Director, stated that since DANIEL did not have medical conditions meriting a medical transfer, there was no need for transfer to a Level 4 facility; further, that if he were to be transferred to KNSPC for administrative reasons, he would be evaluated upon arrival like any other patient.²¹⁷ During an interview with ERAU, Dr. [REDACTED] elaborated on her role in the efforts to transfer DANIEL, stating she started sending emails expressing her concern about DANIEL's lengthy stay in the RHU in September 2023.²¹⁸ She said her last communication regarding an alternative placement for DANIEL was on March 4, 2024, three days before his death.²¹⁹

On February 2, 2024, PA-C [REDACTED] saw DANIEL in his cell²²⁰, DANIEL having refused a scheduled provider appointment the day before. PA [REDACTED] noted the following:

- DANIEL requested a replacement ACE™ wrap for right lower leg edema. He denied leg pain and numbness and was able to bear weight and ambulate without issue.
- DANIEL continued to refuse blood pressure checks, stating they were harmful to his heart. His other vital signs were within normal limits as follows: temperature 97.5 degrees, respirations 16 breaths per minute, heart rate 86 beats per minute, and oxygen saturation 99 percent. He weighed 180.4 pounds.
- DANIEL refused to have the mass on his neck surgically removed. He denied any new or worsening symptoms relating to the mass and said he was able to swallow his food and denied breathing problems.
- DANIEL's external ear canal was red and macerated²²¹, without discharge. The tympanic membrane²²² was scarred. DANIEL said he continued to place a foreign body in the ear because he believed ionized radiation entered his head through the right ear and harmed his brain.

PA-C [REDACTED] diagnosed DANIEL with unspecified otitis externa²²³ of the right ear and ordered ofloxacin 0.3 percent, five drops once a day for seven days. He replaced ACE™ wraps as DANIEL requested. PA-C [REDACTED] encouraged DANIEL to be compliant with appointments

²¹⁶ See [REDACTED] email, dated January 10, 2024. *File path: Medical Records Request-NWIPC (SEA), Emails, Re_Assist with patient transfer.msg.*

²¹⁷ See [REDACTED] email, dated January 16, 2024. *File path: Medical Records Request-NWIPC (SEA), Emails, FW Transfer request (Daniel).msg.*

²¹⁸ ERAU interview with Dr. [REDACTED] on April 2, 2024.

²¹⁹ Dr. [REDACTED] March 4, 2024 was not included among the email correspondence provided by HSA [REDACTED]

²²⁰ See encounter titled request Bandage in HRHU, dated February 2, 2024. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt1.pdf, pages 160-161.*

²²¹ Macerated is skin that looks wrinkly.

²²² Also known as eardrum, the tympanic membrane is a thin, circular layer of tissue that separates the outer ear from the middle ear.

²²³ Otitis externa is inflammation of the ear canal, also known as swimmer's ear.

and medical recommendations. He also encouraged DANIEL to protect his ear from moisture and avoid scratching it.

On March 5, 2024, during a daily SMI round, Dr. [REDACTED] noted DANIEL was a little concerned about his throat.²²⁴ He said that when he sings all the poison comes up in his throat and makes him feel hoarse.

March 7, 2024 - Day of Death

The events described below are based on video footage from fixed security cameras within the RHU, a handheld video camera recording with audio, staff interviews, and incident statements. According to [REDACTED], Facility Investigator, there is a two-to-two-and-a-half-minute difference between the time stamp on the footage and the actual time.²²⁵ The video recording from the handheld camera is not time stamped. Times cited below reflect the time stamp on video from the fixed camera system.

At 7:46 a.m., [REDACTED] RN, conducted nursing rounds in the RHU, stopping briefly at DANIEL's cell.²²⁶ DANIEL did not report any medical, dental, or mental health needs.²²⁷ During an interview with ERAU, RN [REDACTED] stated that when she got to his cell, DANIEL was awake and sitting at the sink; the faucet was running and he was washing his face.²²⁸ RN [REDACTED] reported she said good morning to DANIEL and he responded to her by stating, good morning.

[REDACTED] and [REDACTED] Detention Officers, were assigned to the RHU during the 7:00 a.m. to 3:00 p.m. shift.²²⁹ At 7:56 a.m., Officer [REDACTED] conducted a health and welfare check.²³⁰ Officer [REDACTED] conducted health and welfare checks at 8:19 a.m. and 8:44 a.m. At 8:50 a.m., DO [REDACTED] informed Officer [REDACTED] that during his 8:44 a.m. check, he saw DANIEL leaning over the toilet and there was vomit in his sink.²³¹ The officers agreed they should call medical because DANIEL's behavior was out of character.²³² Officer [REDACTED]

²²⁴See encounter titled request Daily SMI, dated March 5, 2024. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt1.pdf, pages 15-16.*

²²⁵ ERAU interview with Facility Investigator [REDACTED] on April 4, 2024.

²²⁶ See RHU fixed camera video. *File path: General Records Request – NWIPC (SEA), RHU Video, 3-7-24, 0000-1600.avi.*

²²⁷ See encounter titled SEG ROUNDS, dated March 7, 2024. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt1.pdf, pages 5-6.*

²²⁸ ERAU interview with RN [REDACTED] on April 4, 2024.

²²⁹See Shift Rosters. *File path: General Records Request- NWIPC (SEA), Rosters, 3-7-24 GEO Roster Report with Counts.xls, rows 75-76, 192-195, and 281-282.*

²³⁰ See RHU fixed camera video. *File path: General Records Request – NWIPC (SEA), RHU Video, 3-7-24, 0000-1600.avi.*

²³¹ ERAU interview with Officer [REDACTED] on March 29, 2024. See Memorandum by DO [REDACTED] dated March 7, 2024. *File path: General Records Request – NWIPC (SEA), Daniel death report reduced.pdf, page 16.*

²³² ERAU interview with Officer [REDACTED] on March 29, 2024, and DO [REDACTED] on April 2, 2024.

called the medical department to speak with PA-C (b)(6),(b)(7)(C),(b)(7)(F).²³³ (b)(6),(b)(7)(C),(b)(7)(F) Detention Officer assigned to the medical post, answered the phone and told her PA-C (b)(6),(b)(7)(C),(b)(7)(F) was with a patient and he would have him call her back. At 9:50 a.m., Officer (b)(6),(b)(7)(C),(b)(7)(F) called Officer (b)(6),(b)(7)(C),(b)(7)(F) and handed the phone to PA-C (b)(6),(b)(7)(C),(b)(7)(F). Officer (b)(6),(b)(7)(C),(b)(7)(F) described DANIEL's symptoms, including that he had thrown up and clearly was not feeling well.²³⁴ She also mentioned that the growth on DANIEL's neck was bigger. According to Officer (b)(6),(b)(7)(C),(b)(7)(F), PA-C (b)(6),(b)(7)(C),(b)(7)(F) told her the growth was benign and that DANIEL refused treatment. She asked PA-C (b)(6),(b)(7)(C),(b)(7)(F) if DANIEL's condition may be life threatening because she did not want DANIEL to die on her shift. She said PA-C (b)(6),(b)(7)(C),(b)(7)(F) told her if that happened, she could give DANIEL's eulogy. She asked what PA-C (b)(6),(b)(7)(C),(b)(7)(F) would like her to do about DANIEL and he told her that if he passed out, she should call Information Management System (IMS).²³⁵

ERAU asked PA-C (b)(6),(b)(7)(C),(b)(7)(F) about his conversation with Officer (b)(6),(b)(7)(C),(b)(7)(F) during an interview. PA-C (b)(6),(b)(7)(C),(b)(7)(F) recalled the officer said DANIEL was trying to cough or vomit something and that she wanted him to come to the RHU.²³⁶ He did not hear urgency in her words and remarked that vomiting by itself is not necessarily an urgent symptom. He asked Officer (b)(6),(b)(7)(C),(b)(7)(F) if she wanted to call IMS because even if he reported to the RHU, he could not treat DANIEL there; further, based on experience, he anticipated DANIEL would refuse to come medical. PA (b)(6),(b)(7)(C),(b)(7)(F) said he thought it significant that Officer (b)(6),(b)(7)(C),(b)(7)(F) did not choose to call IMS at that point. He told her to do so if things got worse.

After speaking with PA-C (b)(6),(b)(7)(C),(b)(7)(F), DO (b)(6),(b)(7)(C),(b)(7)(F) went to DANIEL's cell to see if he was okay.²³⁷ He told her he ate some pudding the night before and vomited in his sink. DANIEL refused to go to medical and stated that he was fine after vomiting the pudding.

At 8:54 a.m., (b)(6),(b)(7)(C),(b)(7)(F) Captain, made rounds in the unit and passed by DANIEL's cell.²³⁸ Captain (b)(6),(b)(7)(C),(b)(7)(F) told ERAU he was sitting on his bunk with green underwear on his head, and that he smiled and waved to her.²³⁹ Surveillance video footage shows several other staff in the RHU by DANIEL's cell before the emergency.²⁴⁰

²³³ ERAU interview with Officers (b)(6),(b)(7)(C),(b)(7)(F) and (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024. See Memorandum by DO (b)(6),(b)(7)(C),(b)(7)(F) dated March 7, 2024. File path: General Records Request – NWIPC (SEA), Daniel death report_reduced.pdf, pages 10-13.

²³⁴ ERAU interview with Officer (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024. See Memorandum by Officer (b)(6),(b)(7)(C),(b)(7)(F) dated March 7, 2024. File path: General Records Request – NWIPC (SEA), Daniel death report_reduced.pdf, pages 10-15.

²³⁵ IMS is the term used for any emergent need for medical care. File path: Medical Records Request- NWIPC (SEA), NWIPC 2022.02.24 Medical Annual Training, pages 22-24.

²³⁶ ERAU interview with PA-C (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024.

²³⁷ ERAU interview with DO (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024. See Memorandum by DO (b)(6),(b)(7)(C),(b)(7)(F) dated March 7, 2024. File path: General Records Request – NWIPC (SEA), Daniel death report_reduced.pdf, pages 10-13.

²³⁸ See RHU stationary video File path: General Records Request – NWIPC, RHU Video, 3-7-24, 0000-1600.avi.

²³⁹ ERAU interview with Captain (b)(6),(b)(7)(C),(b)(7)(F) on March 29, 2024. See RHU fixed camera video. File path: General Records Request – NWIPC (SEA), RHU Video, 3-7-24, 0000-1600.avi.

²⁴⁰ See RHU stationary video File path: General Records Request – NWIPC, RHU Video, 3-7-24, 0000-1600.avi.

At 9:09 a.m., (b)(6),(b)(7)(C),(b)(7)(F) Detention Officer, conducted a health and welfare check.²⁴¹ DANIEL was in his cell, sitting upright and in no apparent distress.²⁴²

At 10:09 a.m., (b)(6),(b)(7)(C),(b)(7)(F) Detention Officer, conducted a health and welfare check.²⁴³ DANIEL's back was to her but she observed he was sitting on his bed, moving his feet, relaxing and in no apparent distress.²⁴⁴

At 10:13 a.m., Officer (b)(6),(b)(7)(C),(b)(7)(F) went to DANIEL's cell and spoke with him for approximately one minute.²⁴⁵

At 10:22 a.m., LCSW (b)(6),(b)(7)(C),(b)(7)(F), LCSW, conducted an SMI round.²⁴⁶ DANIEL's appearance was well-groomed, and he had a pleasant demeanor. DANIEL reported that he was resting because they poisoned his pudding. During an interview with ERAU, LCSW (b)(6),(b)(7)(C),(b)(7)(F) said DANIEL appeared the same as any other day and showed no emotional or physical distress.²⁴⁷ She asked him if he wanted to see medical and he said he was just going to rest.

At 10:46 a.m., during his routine health and welfare check²⁴⁸, Officer (b)(6),(b)(7)(C),(b)(7)(F) looked in DANIEL's cell and observed DANIEL slumped over on his bed.²⁴⁹ He called to DANIEL and knocked on the door. When DANIEL did not respond, Officer (b)(6),(b)(7)(C),(b)(7)(F) quickly walked to the end of the range and called for Officer (b)(6),(b)(7)(C),(b)(7)(F).²⁵⁰ Officer (b)(6),(b)(7)(C),(b)(7)(F) promptly responded to DANIEL's cell, looked inside, and immediately called IMS over the radio, stating detainee down, and asked the Control Center Officer to open DANIEL's cell door.²⁵¹ Officer (b)(6),(b)(7)(C),(b)(7)(F) entered the cell first, followed by Officer (b)(6),(b)(7)(C),(b)(7)(F).²⁵² DANIEL's lower body and legs were on

²⁴¹ See RHU fixed camera video. *File path: General Records Request – NWIPC (SEA), RHU Video, 3-7-24, 0000-1600.avi.* Officer (b)(6),(b)(7)(C),(b)(7)(F) was relieving Officer (b)(6),(b)(7)(C),(b)(7)(F) for his break.

²⁴² ERAU interview with Officer (b)(6),(b)(7)(C),(b)(7)(F) on April 4, 2024.

²⁴³ See RHU fixed camera video. *File path: General Records Request-NWIPC (SEA), RHU Video, 3-7-24, 0000-1600.avi.* Officer (b)(6),(b)(7)(C),(b)(7)(F) was relieving Officer (b)(6),(b)(7)(C),(b)(7)(F) who was called upon to perform notary services in the law library.

²⁴⁴ ERAU interview with Officer (b)(6),(b)(7)(C),(b)(7)(F) on April 3, 2024.

²⁴⁵ See RHU fixed camera video. *File path: General Records Request-NWIPC (SEA), RHU Video, 3-7-24, 0000-1600.avi.*

²⁴⁶ See encounter titled request Daily SMI, dated March 7, 2024. *File path: Medical Records Request-NWIPC (SEA), Full Medical Record, DANIEL, CHARLES Complete Medical Record Pt1.pdf, pages 3-4.*

²⁴⁷ ERAU interview with LCSW (b)(6),(b)(7)(C),(b)(7)(F) on April 4, 2024.

²⁴⁸ See RHU fixed camera video. *File path: General Records Request-NWIPC (SEA), RHU Video, 3-7-24, 0000-1600.avi.*

²⁴⁹ ERAU interview with Officer (b)(6),(b)(7)(C),(b)(7)(F) on March 29, 2024.

²⁵⁰ ERAU interview with Officer (b)(6),(b)(7)(C),(b)(7)(F) on March 29, 2024, and DC (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024.

²⁵¹ ERAU interview with Officer (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024.

²⁵² See RHU fixed camera video. *File path: General Records Request-NWIPC (SEA), RHU Video, 3-7-24, 0000-1600.avi.*

the bed and his upper torso and head were slumped over, off the bed.²⁵³ Officer (b)(6),(b)(7)(C),(b)(7)(F) performed a sternum rub²⁵⁴ and tried to get a pulse but could not get DANIEL to wake up.²⁵⁵

At 10:48:37 a.m., (b)(6),(b)(7)(C),(b)(7)(F) and (b)(6),(b)(7)(C),(b)(7)(F), Detention Officers, arrived at the cell.²⁵⁶ Officer (b)(6),(b)(7)(C),(b)(7)(F) entered the cell and Officer (b)(6),(b)(7)(C),(b)(7)(F) remained in the doorway; Officer (b)(6),(b)(7)(C),(b)(7)(F) stepped out of the cell. (b)(6),(b)(7)(C),(b)(7)(F) Detention Officer, arrived at 10:48:53 and went into the cell. At 10:49:00 a.m., (b)(6),(b)(7)(C),(b)(7)(F) Detention Officer, arrived and began recording with a handheld video camera from the top of the stairs and then moved directly outside DANIEL's cell.²⁵⁷ Video from the handheld camera shows Officers (b)(6),(b)(7)(C),(b)(7)(F) and (b)(6),(b)(7)(C),(b)(7)(F) moved DANIEL off the bed and onto the floor into a prone position, Officer (b)(6),(b)(7)(C),(b)(7)(F) holding DANIEL's legs and Officer (b)(6),(b)(7)(C),(b)(7)(F) supporting his head.²⁵⁸ Once he was safely on the floor, Officer (b)(6),(b)(7)(C),(b)(7)(F) is heard stating over the radio that medical was needed.

At 10:49:25 Lieutenant (b)(6),(b)(7)(C),(b)(7)(F) entered the cell.²⁵⁹ He told Officer (b)(6),(b)(7)(C),(b)(7)(F) and (b)(6),(b)(7)(C),(b)(7)(F) that they needed to turn DANIEL over and the three of them moved him to a supine position while continually calling his name.²⁶⁰ Lieutenant (b)(6),(b)(7)(C),(b)(7)(F) instructed that they watch his head as there was limited space between the bunk and the desk. As they were rolling DANIEL over Officer (b)(6),(b)(7)(C),(b)(7)(F) left the cell and at 10:49:50 a.m., (b)(6),(b)(7)(C),(b)(7)(F) RN, entered. One of the staff members is heard stating DANIEL was not breathing.

(b)(6),(b)(7)(C),(b)(7)(F) Lieutenant, entered the cell at 10:50:15 and asked if DANIEL was breathing or had a pulse.²⁶¹ Officer (b)(6),(b)(7)(C),(b)(7)(F) said he felt a pulse. At 10:50:18 a.m., PA-C (b)(6),(b)(7)(C),(b)(7)(F) arrived on the second tier of the RHU and placed the red emergency bag on the floor. He entered the cell at 10:50:20 and at 10:50:22 a.m., (b)(6),(b)(7)(C),(b)(7)(F) LPN, arrived with an AED. PA-C (b)(6),(b)(7)(C),(b)(7)(F) pulled DANIEL on the floor into the hallway immediately outside the cell. (b)(6),(b)(7)(C),(b)(7)(F) RN, arrived in the unit with a gurney and oxygen tank.²⁶²

²⁵³ ERAU interview with Officer (b)(6),(b)(7)(C),(b)(7)(F) on March 29, 2024.

²⁵⁴ A sternum rub is application of painful stimulus with the knuckles of a closed fist to the center of the chest of a patient who is not alert and does not respond to verbal stimuli.

²⁵⁵ ERAU interview with Officer (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024.

²⁵⁶ See RHU fixed camera video. File path: General Records Request-NWIPC (SEA), RHU Video, 3-7-24, 0000-1600.avi.

²⁵⁷ The handheld video camera also recorded audio.

²⁵⁸ See handheld camera video. File path: General Records Request-NWIPC (SEA), Handheld Camera Footage, 3.7.2024 response in RHU.MP4.

²⁵⁹ See RHU fixed camera video. File path: General Records Request-NWIPC (SEA), RHU Video, 3-7-24, 0000-1600.avi.

²⁶⁰ See handheld camera video. File path: General Records Request-NWIPC (SEA), Handheld Camera Footage, 3.7.2024 response in RHU.MP4.

²⁶¹ Id.

²⁶² See RHU fixed camera video. File path: General Records Request-NWIPC (SEA), RHU Video, 3-7-24, 0000-1600.avi.

At 10:50:56 a.m., PA-C (b)(6),(b)(7)(C),(b)(7)(F) said he could not get a pulse; call 911, as he started performing chest compressions.²⁶³ Lieutenant (b)(6),(b)(7)(C),(b)(7)(F) immediately used his radio to call the control center and directed them to call 911. The Control officer acknowledged and logged the time of the call to EMS was 10:44 a.m.²⁶⁴ In the meantime, Officer (b)(6),(b)(7)(C),(b)(7)(F) checked DANIEL's cell and he did not find anything DANIEL could have taken.²⁶⁵

At 10:51:35 a.m., PA-C (b)(6),(b)(7)(C),(b)(7)(F) asked for the Ambu bag.²⁶⁶ RN (b)(6),(b)(7)(C),(b)(7)(F) placed the Ambu bag over DANIEL's mouth at 10:52:18 a.m., attempted to push air, and repositioned DANIEL's head.²⁶⁷ PA-C (b)(6),(b)(7)(C),(b)(7)(F) requested the AED and Officer (b)(6),(b)(7)(C),(b)(7)(F) took over compressions while PA-C (b)(6),(b)(7)(C),(b)(7)(F) assisted RN (b)(6),(b)(7)(C),(b)(7)(F) with the Ambu bag.

At 10:53:22 LPN (b)(6),(b)(7)(C),(b)(7)(F) placed the AED pads on DANIEL while PA-C (b)(6),(b)(7)(C),(b)(7)(F) and RN (b)(6),(b)(7)(C),(b)(7)(F) continued to work on establishing an airway.²⁶⁸ At 10:54:04 the AED delivered a shock, then Officer (b)(6),(b)(7)(C),(b)(7)(F) resumed compressions. He stopped compressions 30 seconds later while PA-C (b)(6),(b)(7)(C),(b)(7)(F) and RN (b)(6),(b)(7)(C),(b)(7)(F) continued to try to deliver air with the Ambu bag. Compressions resumed at 10:54:40.

At 10:55:05 RN (b)(6),(b)(7)(C),(b)(7)(F) took over chest compressions while PA-C (b)(6),(b)(7)(C),(b)(7)(F) and RN (b)(6),(b)(7)(C),(b)(7)(F) continued to work on the airway.²⁶⁹ A minute later, one of them is heard stating nothing is going in. The AED directed giving breaths, but PA-C (b)(6),(b)(7)(C),(b)(7)(F) and RN (b)(6),(b)(7)(C),(b)(7)(F) were unable to get air into DANIEL's lungs. His chest did not rise.

At 10:56:36 (b)(6),(b)(7)(C),(b)(7)(F) PA-C, took over chest compressions while medical staff were palpating DANIEL's throat. Someone remarked that they did not see anything in his mouth when they looked, presumably referring to an object that may be obstructing the airway. At 10:56:57 the AED analyzed and determined no shock was advised. PA-C (b)(6),(b)(7)(C),(b)(7)(F) resumed compressions at 10:57:17. During an interview with ERAU, PA-C (b)(6),(b)(7)(C),(b)(7)(F) stated the mass on DANIEL's neck inhibited establishment of an airway.²⁷⁰

²⁶³ See handheld camera video. File path: General Records Request-NWIPC (SEA), Handheld Camera Footage, 3.7.2024 response in RHU.MP4.

²⁶⁴ Based on video documentation, the time entered by the Control officer appears to be in error.

²⁶⁵ ERAU interview with DO (b)(6),(b)(7)(C),(b)(7)(F) on April 4, 2024.

²⁶⁶ Ambu bag is a medical tool which forces air into the lungs of patient who are not breathing.

²⁶⁷ See handheld camera video. File path: General Records Request-NWIPC (SEA), Handheld Camera Footage, 3.7.2024 response in RHU.MP4.

²⁶⁸ See handheld camera video. File path: General Records Request-NWIPC (SEA), Handheld Camera Footage, 3.7.2024 response in RHU.MP4.

²⁶⁹ See handheld camera video. File path: General Records Request-NWIPC (SEA), Handheld Camera Footage, 3.7.2024 response in RHU.MP4.

²⁷⁰ ERAU interview with (b)(6),(b)(7)(C),(b)(7)(F) PA, on April 2, 2024.

At 10:58:23 a.m., Tacoma Fire Department (TFD) EMS arrived at DANIEL's side.²⁷¹ PA-C (b)(6),(b)(7)(C),(b)(7)(F) briefed the EMS staff while PA-C (b)(6),(b)(7)(C),(b)(7)(F) continued chest compressions and RN Moffett continued to try and get air into DANIEL. PA-C (b)(6),(b)(7)(C),(b)(7)(F) informed EMS responders that DANIEL had a mass on his throat and that he did not know if it was obstructing the airway. The lead responder asked if TIPC medical staff were able to ventilate DANIEL and PA-C (b)(6),(b)(7)(C),(b)(7)(F) said they were not. EMS personnel took over chest compressions at 10:58:40 a.m. and ventilation at 10:59:05. EMS personnel attempted to intubate DANIEL, but they were initially unsuccessful.²⁷² Eventually they were able to introduce air by lifting DANIEL's head and tilting it to the left. At 11:37:18 a.m., the lead EMS responder walked down the corridor with his phone in his hand and returned to the group at 11:41:25 a.m., at which time EMS terminated CPR.²⁷³ Dr. (b)(6),(b)(7)(C),(b)(7)(F) at Good Samaritan Hospital authorized termination of lifesaving efforts.²⁷⁴ EMS personnel covered DANIEL with a blanket when they discontinued CPR, and they departed the area at 11:47 a.m.²⁷⁵

At 11:59 a.m., Officer (b)(6),(b)(7)(C),(b)(7)(F) placed a privacy screen in the hallway to block the view of DANIEL's body from other detainees. RN (b)(6),(b)(7)(C),(b)(7)(F) and Officer (b)(6),(b)(7)(C),(b)(7)(F) remained on the RHU upper tier to maintain observation of DANIEL's body.

At 12:22 p.m., Lieutenant (b)(6),(b)(7)(C),(b)(7)(F) and two officers from the Tacoma Police Department arrived on scene and at 12:59 p.m., the Tacoma Police Department Forensic Team arrived.

At 1:34 p.m., the Medical Examiner arrived. After local forensic detectives from the Tacoma Police Department and the Medical Examiner took photographs, they placed DANIEL's body in a body bag and onto a flexible gurney. With Lieutenant (b)(6),(b)(7)(C),(b)(7)(F) assistance, they carried DANIEL's body out of the RHU at 2:02 p.m. and placed the body on the Medical Examiner's gurney. The Medical Examiner placed the body in his vehicle.²⁷⁶

Post-Death Actions

²⁷¹ See handheld camera video. File path: General Records Request-NWIPC (SEA), Handheld Camera Footage, 3.7.2024 response in RHU.MP4.

²⁷² See Prehospital Care Report, dated March 7, 2024. File path: Medical Records Request-NWIPC (SEA), Tacoma Fire Dept Record, Daniel, Charles – Tacoma Fire Dept.pdf., page 1.

²⁷³ See RHU fixed camera video. File path: General Records Request – NWIPC, RHU Video, 3-7-24, 0000-1600.avi.

²⁷⁴ See Prehospital Care Report, dated March 7, 2024. The report does not document the time Dr. (b)(6),(b)(7)(C),(b)(7)(F) authorized termination of lifesaving measures. File path: Medical Records Request-NWIPC (SEA), Tacoma Fire Dept Record, Daniel, Charles – Tacoma Fire Dept.pdf., page 1.

²⁷⁵ See RHU fixed camera video. File path: General Records Request – NWIPC, RHU Video, 3-7-24, 0000-1600.avi.

²⁷⁶ See handheld camera video. File path: General Records Request - NWIPC, Handheld Camera Footage, 3.7.2024 RHU camera 5.MP4.

On March 7, 2024, at 2:27 p.m.,²⁷⁷ Sergeant (b)(6),(b)(7)(C),(b)(7)(F) and DO (b)(6),(b)(7)(C),(b)(7)(F) returned to DANIEL's cell, made a video recording of DANIEL's cell and property,²⁷⁸ and inventoried his personal property.²⁷⁹ ICE ERO Supervisory Detention and Deportation Officer (b)(6),(b)(7)(C),(b)(7)(F) signed for DANIEL's property and placed it into evidence.²⁸⁰

During interviews, ERAU asked staff members if they were offered supportive counseling following DANIEL's death. Most staff members stated Employee Assistance Program (EAP) services were offered to them; some staff were not aware of EAP.

CONCLUSIONS

Creative Corrections identified the following deficiency in the ICE 2011 PBNDS, revised 2016.

Medical Care, Section (AA)(4), states "Prior to administration of psychotropic medications, a separate documented informed consent, that includes a description of the medication's side effects, shall be obtained."

- On April 3, 2020, PA-C (b)(6),(b)(7)(C),(b)(7)(F) did not obtain DANIEL's consent for psychotropic medication, fluoxetine, and did not document she discussed the medication's side effects.

²⁷⁷ See RHU fixed camera video. File path: General Records Request-NWIPC (SEA), RHU Video, 3-7-24, 0000-1600.avi.

²⁷⁸ See Sergeant (b)(6),(b)(7)(C),(b)(7)(F) property documentation video. File path: General Records Request-NWIPC (SEA), Property, 03.07.2024 Sgt. (b)(6),(b)(7)(C),(b)(7)(F) property documentation.MP4.

²⁷⁹ See DANIEL's Property Inventory form signed by Sergeant (b)(6),(b)(7)(C),(b)(7)(F). File path: General Records Request – NWIPC, Property, Daniel A022 745 951 Property Inventory.pdf.

²⁸⁰ See Property Inventory Form and photographs. File path: ERO Records Request, Daniel A022 745 951 Property Inventory.pdf and Daniel A022 745 951, Property Picture.jpg.